SPRING 2023

Institute of Politics Policy Program
Housing Spring Policy Team

A Directed Inventory of Federal Homelessness Research

Prepared for:
The U.S. Interagency Council on Homelessness
Program and Management Analyst Alec Vandenburg

Team Leads: Ethan Hsiao, Javier Serra, Michelle Amponsah

Researchers: Bradley Shearer, Nghia Nguyen, Briana Howard, Ogechi Obi, Michelle Jiang, Eric Forteza Romero, Darian Benitez Sanchez

Policy Program Leadership: Samantha Williams and Andrew Charroux

HARVARD Kennedy School
INSTITUTE OF POLITICS

IOP POLICY PROGRAM

UNITED STATES INTERAGENCY COUNCIL ON HOMELESSNESS

ABOUT THE INSTITUTE OF POLITICS POLICY PROGRAM (IOP)

The Institute of Politics is a nonprofit organization located in the John F. Kennedy School of Government at Harvard University. It is a living memorial to President John F. Kennedy, and its mission is to unite and engage students, particularly undergraduates, with academics, politicians, activists, and policymakers on a nonpartisan basis and to stimulate and nurture their interest in public service and leadership. The Institute strives to promote greater understanding and cooperation between the academic world and the world of politics and public affairs. Led by a Director, Senior Advisory Board, Student Advisory Committee, and staff, the Institute provides wide-ranging opportunities for both Harvard students and the general public. This report is the result of a semester-long intensive effort by a team of undergraduates. The Institute of Politics does not endorse specific policy positions; accordingly, all views expressed in this publication should be understood to be solely those of the authors.

ABOUT THE U.S. INTERAGENCY COUNCIL ON HOMELESSNESS

The U.S. Interagency Council on Homelessness (USICH) is the sole federal agency dedicated to eradicating homelessness in the United States. In accordance with the belief that housing is a right, USICH works with federal, state, and local government to aid communities in more efficient and equitable ways.

2023 by the Institute of Politics. All rights reserved.
Institute of Politics
Harvard University
79 John F. Kennedy Street
Cambridge, MA 02138
Tel: (617) 495-1360
Fax: (617) 496-4344
Web: www.iop.harvard.edu
Email: ioppolicy@gmail.com

Photo from https://imla.org/2021/04/los-angeles-is-ordered-to-provide-shelter-for-entire-skid-row-homeless-population-appeal-filed/

Table of Contents

Table of Contents 2
Executive Summary 8
Primary Inventory 9
Subject Tags 9
Department of Education 10
Literature 10
Comprehensive Challenges for the Well Being of Young Children: A Population-based Study of Publicly Monitored Risks in a Large Urban Center 10
Cumulative Adversity Profiles among Youth Experiencing Housing and Parental Care Instability 10
Educating Homeless and Highly Mobile Students: Implications of Research on Risk and Resilience 11
Going Without: An Exploration of Food and Housing Insecurity Among Undergraduates 12
Identified Student Homelessness in Camden City Schools, 2014-15 through 2018-19 12
Overcoming the "Trash Talk in Your Head": Extending an Ethic of Care to Students Experiencing Intersectional Stigma in Community College 13
Student Homelessness in Camden City School District: Mobility and Frequency 13
The Impacts of Homelessness Prevention and Rapid Rehousing on Homeless and Highly Mobile Students 14
Summary of Findings 15
Department of Health and Human Services 17
Literature 17
Adolescent Well-Being after Experiencing Family Homelessness 17
Behavioral Health Improvements Over Time among Adults in Families Experiencing Homelessness 18
Child Separation among Families Experiencing Homelessness 19
Comorbid Health Conditions and Treatment Utilization Among Individuals with Opioid Use Disorder Experiencing Homelessness 19
Establishing Eligibility for SSI for Chronically Homeless People 20
Evaluability Assessment of Discharge Planning and the Prevention of Homelessness 20
Exploring Homelessness Among People Living in Encampments and Associated Costs 21
Final Report for the Independent Evaluation of the Substance Abuse Prevention and Treatment Block Grant Program 21
Health Conditions Among Individuals with a History of Homelessness Brief 22
Health, Housing, and Service Supports for Three Groups of People Experiencing

Chronic Homelessness 22
Hispanic Families Experiencing Homelessness 23
Housing Assistance for Youth Who Have Aged Out of Foster Care: The Role of the Chafee Foster Care Independence Program 24
Homeless Caseload is Associated with Behavioral Health and Case Management Staffing in Health Centers 25
Identifying and Serving LGBTQ Youth: Case Studies of Runaway and Homeless Youth Program Grantees 26
Incarceration and Homelessness 27
Individuals Experiencing Homelessness are Likely to have Medical Conditions Associated with Severe Illness from COVID-19 27
Lessons from the Field: Using Housing Vouchers to Support Youth with Child Welfare Experience at Risk of Homelessness 28
Leveraging Data to Support Children Experiencing Homelessness 28
Medicaid and Permanent Supportive Housing for Chronically Homeless Individuals: Literature Synthesis and Environmental Scan 30
Medicaid and Permanent Supportive Housing for Chronically Homeless Individuals: Emerging Practices From the Field 31
Mental Health of Vulnerable Youth and their Transition to Adulthood: Examining the Role of the Child Welfare, Juvenile Justice, and Runaway/Homeless Systems 31
Potential Analyses with Homelessness Data: Ideas for Policymakers and Researchers 32
Predicting Repeated and Persistent Family Homelessness: Do Families' Characteristics and Experiences Matter 33
Pretesting a Human Trafficking Screening Tool in the Child Welfare and Runaway and Homeless Youth Systems 34
Promising Practices for Children Experiencing Homelessness: A Look at Two States 34
Public Housing Agencies and Permanent Supportive Housing for Chronically Homeless People 35
State Strategies for Coordinating Medicaid Services and Housing for Adults with Behavioral Health Conditions 36
State Strategies for Improving Provider Collaboration and Care Coordination for Medicaid Beneficiaries with Behavioral Health Conditions 37
Strategic Action Plan on Homelessness 38
Study of HHS Programs Serving Human Trafficking Victims 39
The Implementation of Maternity Group Home Programs: Serving Pregnant and Parenting Teens in a Residential Setting 39
The 1996 National Survey of Homeless Assistance Providers and Clients: A Comparison of Faith-Based and Secular Non-Profit Programs 40
Well-being of Young Children after Experiencing Homelessness 41

Summary of Findings 43
Department of Housing and Urban Development 44
Literature 44
Addressing Housing Affordability in High-Cost Metropolitan Areas in the United States 44
American Housing Survey: Housing Adequacy and Quality As Measured by the AHS 44
A Picture of Disability and Designated Housing 45
Assessment Tools for Allocating Homelessness Assistance: State of the Evidence 45
A Qualitative Assessment of Parental Preschool Choices and Challenges Among Families Experiencing Homelessness 46
Characteristics of HUD-Assisted Renters and Their Units in 2003 47
Chicago, Illinois Community Encampments Report 47
Child and Partner Transitions among Families Experiencing Homelessness 48
Choice Matters: Housing Models that May Promote Recovery for Individuals and Families Facing Opioid Use Disorder 49
Consumer Integration and Self-Determination in Homelessness Research, Policy, Planning, and Services 49
Continuity and Change: Demographic, Socioeconomic, and Housing Conditions of American Indians and Alaska Natives 50
Developing Choice Neighborhoods: An Early Look at Implementation in Five Sites - Interim Report 51
Domestic Violence Housing First (DVHF) Demonstration Evaluation 51
Employment of Families Experiencing Homelessness 52
End of Participation in Assisted Housing: What Can We Learn About Aging in Place? 53
Evaluation of the HUD-DOJ Pay for Success Permanent Supportive Housing Demonstration Baseline Report 53
Evaluation of the HUD Youth Homelessness Demonstration Program 54
Family Options Study: 3-Year Impacts of Housing and Services Interventions for Homeless Families 55
Family Options 12-Year Study 55
Family Unification Program - Family Self-Sufficiency (FUP-FSS) Demonstration Evaluation 56
Homelessness Prevention Study - Prevention Programs Funded by the Homelessness Prevention and Rapid Re-Housing Program 57
Housing for Youth Aging Out of Foster Care 57
Housing Problems and Needs of Native Hawaiians 58
Houston, Texas: Community Encampment Report 59
HUD Investments in Impoverished Areas for House Report 116-106 59

Implementing Approaches to Address Unsheltered Homelessness 60
Improving the Coordination of Services for Adults with Mental Health and Substance Use Disorders: Profiles of Four State Medicaid Initiatives 61
Issue Brief: Evaluating Housing Programs for Youth who Age out of Foster Care 61
Market Predictors of Homelessness: How Housing and Community Factors Shape Homelessness Rates Within Continuums of Care 62
Patterns of Benefit Receipt among Families who Experience Homelessness 63
Quality Control for Rental Assistance Subsidy Determinations: Final Income Match Report FY 2013 63
Rapid Re-Housing in 2018: Program Features and Assistance Models Understanding Rapid Re-Housing 64
Rental Housing Discrimination on the Basis of Mental Disabilities: Results of Pilot Testing 65
San Jose, California Community Encampment Report 65
Rural Homelessness 66
Section 202 Supportive Housing for the Elderly: Program Status and Performance Measurement 66
Study of Household Transition from the Disaster Housing Assistance Program (DHAP-Katrina): Final Report 67
Tacoma, Washington Community Encampment Report 67
The Applicability of Housing First Models to Homeless Persons with Serious Mental Illness 68
The Bridge to Family Self-Sufficiency (BridgeFSS) Demonstration 69
The Evaluation of the Neighborhood Stabilization Program 69
Understanding Rapid Re-Rousing through Participants' Experiences 70
Understanding Rapid Re-Housing: What Did We Learn? 71
Unsheltered Homelessness and Homeless Encampments in 2019 71
Veterans Homelessness Prevention Demonstration Evaluation Final Report 72
Voices of Youth Count Comprehensive Report: Youth Homelessness in America 72
What Happens to Housing Assistance Leavers? 73
What Happens to Low-Income Housing Tax Credit Properties at Year 15 and Beyond? 74
Worst Case Housing Needs: 2015 Report to Congress 75
Worst Case Housing Needs of People with Disabilities: Supplemental Findings of the Worst Case Housing Needs 2009 Report to Congress 75
Summary of Findings 77
Department of Veteran Affairs 79
Literature 79
A Brief Values Intervention to Support Veterans in Early Buprenorphine Treatment 79
Assessing an Initiative to Facilitate Long-Term Financial and Housing Stability in

Vulnerable Veterans 79
Assessing the Impact of a Pre-visit Readiness Telephone Call on Video Visit Success Rates 79
Capturing the Dynamics of Homelessness through Ethnography and Mobile Technology 80
Comparing Quality of Care between VA-Delivered and VA-Purchased Care: The Need for Better Risk Adjustment 81
Designing a Mobile App to Support Academic Success for Student Veterans 81
Diagnoses Treated in Ambulatory Care Among Homeless-Experienced Veterans: Does Supported Housing Matter? 82
Differences in Experiences With Care Between Homeless and Nonhomeless Patients in Veterans Affairs Facilities With Tailored and Nontailored Primary Care Teams 82
Differential Risk for Homelessness Among US Male and Female Veterans With a Positive Screen for Military Sexual Trauma 83
Does housing chronically homeless adults lead to social integration? 84
Enhancing Community Integration for Homeless Veterans 84
Evaluating Health Services for Justice-Involved Veterans to Improve Policy and Practice 85
Evaluating the impact of dental care on housing intervention program outcomes among homeless veterans 86
Evaluating the VA Make-or-Buy Decision in Emergency Care 86
Examining racial differences in community integration between Black and White homeless veterans 87
Factors contributing to eviction from permanent supportive housing: Lessons from HUD-VASH 87
Healthcare service utilization for formerly homeless veterans in permanent supportive housing: Do neighborhoods matter? 88
"Homelessness and trauma go hand-in-hand": pathways to homelessness among women veterans 88
Implementing a Clinical-Community Partnered Intervention to Address Food Insecurity Among High-Risk Veterans 89
Intimate partner violence, unhealthy alcohol use, and housing instability among women veterans in the Veterans Health Administration. 90
Medical-Legal Partnerships At Veterans Affairs Medical Centers Improved Housing And Psychosocial Outcomes For Vets 90
Medical-legal partnerships to prevent evictions and homelessness among veterans 91
Military Sexual Trauma Among Homeless Veterans 91
Neuroinflammation and Oxidative Stress in Veterans with Schizophrenia 92
One-year incidence and predictors of homelessness among 300,000 U.S. Veterans seen in specialty mental health care 92
Preventing Homelessness Through the National Call Center for Homeless Veterans:

Analysis of Calls and Service Referrals 93
Providing intensive addiction/housing case management to homeless veterans enrolled in addictions treatment: A randomized controlled trial 94
Quality of Mental Health Services for Homeless Veterans in Primary Care Settings 94
Risk Factors for Homelessness Among US Veterans 95
Risk factors for Veteran food insecurity: findings from a National US Department of Veterans Affairs Food Insecurity Screener 95
Social and Behavioral Determinants of Health in High-Risk Veterans 96
Strategies to improve Utilization of Post-overdose Evidence-based Risk mitigation among Non-fatal Overdoses in VA (SUPER NOVA) 96
Substance Use Rates of Veterans with Depression Leaving Incarceration: A Matched Sample Comparison with General Veterans 97
“They’re homeless in a home”: Retaining homeless-experienced consumers in supported housing 97
The Leading Evaluations to Advance VA's Response to National Priorities (LEARN) Evidence-Based Policy Evaluation Center 98
The potential for health-related uses of mobile phones and internet with homeless veterans: results from a multisite survey 99
The Role of Medications in Predicting Suicide-Related Outcomes and Unintended Death in Older Veterans 99
Using Data Analytics and Targeted Whole Health Coaching to Reduce Frequent Utilization of Acute Care among Homeless Veterans 100
VA's expansion of supportive housing: successes and challenges on the path toward Housing First 101
What do Veterans with Homeless Experience Want Us to Know That We Are Not Asking? A qualitative content analysis of comments from a national survey of health care experience 101
Which Homeless Veterans Benefit From a Peer Mentor and How? 102
Summary of Findings 103

Executive Summary

The purpose of this brief is to help advance the creation and implementation of the Biden-Harris Administration's Federal Strategic Plan to End and Prevent Homelessness, which aims to reduce homelessness by 25 percent by 2025. By providing an inventory of recent and current research investments, the inventory serves the federal homelessness research agenda by highlighting important research areas that are overlooked. This brief serves as an inventory of federal homelessness research focusing on four government agencies: the Department of Education, Department of Health and Human Services (HHS) , Department of Housing and Urban Development (HUD), and Department of Veteran's Affairs (VA). The inventory contains the title of the study, project period, funding level, commissioning agency, principal investigators, and a short description of the research, where available. Each Department also contains a summary of findings, which identifies trends in the Department's homelessness literature and gaps in the literature.

Primary Inventory

Subject Tags

The following subject tags are key phrases ('umbrella terms') that categorize the homelessness literature and make navigation of the inventory more efficient. For example: to search for all studies pertaining to Housing_First, press 'Command + F' and type the subject tag 'Housing_First'.

affordable_housing	*housing_market*	*supportive_services*
Continuum_of_Care	*mental_health*	*temporary_housing*
emergency_housing	*physical_health*	*unsheltered_homelessness*
homelessness_prevention	*rapid_rehousing*	*youth_homelessness*
Housing_First	*substance_abuse*	

Department of Education

Literature

Comprehensive Challenges for the Well Being of Young Children: A Population-based Study of Publicly Monitored Risks in a Large Urban Center

Conducted with funding from Institute of Education Sciences, the William Penn Foundation, and the Stoneleigh Foundation with data from 2005-2006. Primary research leads were Heather L. Rouse, John W. Fantuzzo & Whitney LeBoeuf

Subject(s): youth_homelessness

This study investigated the unique and cumulative relations between risks that are monitored by public surveillance systems, such as homelessness, and academic and behavioral outcomes for an entire cohort of third graders in Philadelphia. It concluded that classroom behavior was significantly influenced by early familial and social risks (i.e., teen mother, low maternal education, homelessness, and maltreatment). The researchers called for interagency programs to alleviate these risks that are beyond those associated with traditional academic instruction.

Cumulative Adversity Profiles among Youth Experiencing Housing and Parental Care Instability

Conducted with funding in part by a Eunice Kennedy Shriver National Institute of Child Health and Human Development research infrastructure grant, R24 HD042828 and by an Institute of Education Sciences postdoctoral training grant, R324B180001, to the Prevention Science Institute at the University of Oregon. Conducted in 2019, led by Henry Crumé, Paula S. Nurius, PhD, and Christopher M. Fleming, PhD.

Subject(s): youth_homelessness

This study applies cumulative adversity and stress proliferation theories to examine risk and protective resource profiles of youth with three different levels of housing and parental care instability. The study used data from a state representative sample of school-based adolescent students. The researchers observed more harmful functioning in youth with greater levels of housing and parental instability. In those experiencing instability, higher frequency of victimization, lower grades, diminished self-regulation capabilities and school engagement, weakened psychological health, and strained family and peer relationships. They also identified greater representation of males, sexual minorities, and youth of color in those who experience housing and parental instability.

Educating Homeless and Highly Mobile Students: Implications of Research on Risk and Resilience

Conducted with funding from Center for Urban and Regional Affairs and the Institute of Education with Data from 2003 through 2009 and study published in 2015. Primary research leads were Ann S. Masten, Aria E. Fiat, Madelyn H. Labella, Ryan A. Strack.

Subject(s): youth_homelessness

After summarizing findings from two decades of research on academic risk and resilience in children experiencing homelessness, the researchers found that homeless children share many risks with other disadvantaged children. The research also highlights resilience, with many homeless students succeeding in school. Therefore, early childhood education, screening and access to quality programs combined with outreach and communication, are important for preventing achievement disparities.

Going Without: An Exploration of Food and Housing Insecurity Among Undergraduates

Conducted with data from surveys in 2009 and 2015, and 2015 and 2016. Funding was federal and private, from Institute of Education Sciences Grant No. R305B090009, National Science Foundation Grant No. DGE-1256259, and the Lumina Foundation. Research lead by Katharine M. Broton and Sara Goldrick-Rab.

Subject(s): youth_homelessness

The research showed that at least one-third of 2-year students are housing insecure, with up to 14% being homeless. Between 11% and 19% of 4-year students are housing insecure. Moreover, there is no evidence that the problem of food and housing insecurity has improved over the past 8 years. Researchers therefore called for more research to understand housing and food insecurity, and the creation of interventions to help ensure that students' basic needs are met.

Identified Student Homelessness in Camden City Schools, 2014-15 through 2018-19

Conducted with funding from U.S. Department of Education, Institute of Education Sciences to Cutuli (PI), and Cintron (Co-PI) (R305H190067) spanning 2014 to 2019. Led by J.J. Cutuli, Sandra Cintrn, John Iko, and Aaron Truchil.

Subject(s): youth_homelessness

This research aimed to identify student homelessness in Camden City, New Jersey. They found that "about 1 out of every 20 (4.59%) students was identified as experiencing homelessness" and "annual rates of identified homelessness increased over time" but are likely undercounting the actual amount due to factors like Hurricane Maria in Puerto Rico that brought students to Camden. The report also made recommendations on how to best support unhoused students.

Overcoming the "Trash Talk in Your Head": Extending an Ethic of Care to Students Experiencing Intersectional Stigma in Community College

Conducted with funding from Institute of Education Sciences, U.S. Department of Education, through Grant R305A160166 to Florida State University, and the Bill & Melinda Gates Foundation from fall 2014 to spring 2018, led by Rebecca L. Brower, Tamara Bertrand Jones, Shouping Hu.

Subject(s): supportive_services

This research discusses intersectional stigma experienced by individuals who share both a minoritized identity and a socially stigmatized identity which includes a previous history of homelessness. The researchers concluded that community colleges should increase their efforts to assist students suffering from intersectional stigma through using staff with experience with those same forms of stigma to act as role models.

Student Homelessness in Camden City School District: Mobility and Frequency

Conducted with funding from Institute of Education Sciences under Aaron Truchil.

Subject(s): youth_homelessness

This study describes patterns of homelessness in Camden City School District. Trends discovered across homeless students include that students experiencing homelessness are more likely to enroll late, transfer early, repeat a grade, change school mid-year, and be chronically absent, with these trends increasing for students experiencing multi-year homelessness.

The Impacts of Homelessness Prevention and Rapid Rehousing on Homeless and Highly Mobile Students

Conducted with funding from the Spencer Foundation, the Institute for Education Sciences (R305B140009), and the Anne T. and Robert M. Bass Stanford Graduate Fellowship. Data from January 2014 through September 2017 with paper published in 2020, led by Elise Dizon-Ross.

Subject(s): youth_homelessness

This paper examines the effects of a homelessness prevention and rapid rehousing program—which combines temporary rental subsidies with light-touch case management—on homeless and highly mobile students in a large urban school district. The researchers found that housing interventions significantly reduced the likelihood of students having multiple behavioral incidents in a month, but increased students' absence rates and chronic absenteeism, particularly for students who are rehoused outside the city proper but remain enrolled in the central district. These results highlight both the positive impacts as well as unintended consequences of this intervention.

Summary of Findings

The Department of Education research indicated the predictable impacts that housing insecurity has on a student's education, a surprising downside to rapid rehousing, and an overall resilience amongst homeless children when offered support programs.

First off, housing insecurity often causes chronic absenteeism at school, difficulty in school due to a lack of steady housing to do homework, focus, rest, etc., and behavioral problems often as a result of family dysfunction. The research verified what seems to be intuitive regarding students experiencing homelessness: housing insecurity makes their education far more difficult. This implies that getting students into steady housing is the best way to help their education; however, the literature also showed a downside to rapid rehousing for students.

Oftentimes, students get placed into homes that are far from the current school district making attendance even more difficult. This area could use more research to explore how students can be relocated to areas where they can still be near their school, and if not, possibly having built-in support to help them transition to a school closer by. Furthermore, homeless children show positive responses to resources like early childhood education and other support programs. This suggests that placing youth into schools is crucial, even if they are still dealing with homelessness. Homeless students tend to perform poorly given their circumstances, but research seems to emphasize to policymakers that placing students into school should still be a priority.

Lastly, there was a lack of research on housing insecurities among undergraduate students at the college-aged population. During winter break and the summer, students do not have access to campus housing or their school mailing address, and this period of time has not been specifically covered in the existing Department of Education literature that mostly focuses on

K-12 education. Additional data is necessary to better understand the extent and nature of material hardship among college students. Future research could also uncover ways in which campus policies, personnel and peers either consciously or unconsciously reinforced stigma.

Department of Health and Human Services

Literature

Adolescent Well-Being after Experiencing Family Homelessness

Conducted with funding from HHS under Jessica Thornton Walker, Scott R. Brown, and Marybeth Shinn

Subject(s): youth_homelessness

The research discusses adolescent outcomes after experiencing a shelter stay. Many continue to face housing challenges and instability. Further, these adolescents have higher levels of school mobility (the frequent changing of schools) which is linked to poorer grades and classroom behavior compared to their peers across income levels. Moreover, overcrowding in households holds a similar effect. This research could possibly explain a cycle of poverty in which housing insecurity drives underperformance in academic studies for adolescents. Researchers found that overcrowding also has a negative effect on adolescents who are not experiencing homelessness but for whom quality of housing is still an influential factor.

Are Homeless Families Connected to the Social Safety Net?

Conducted with funding from HUD in the Family Options Study in partnership with HHS, the Office of Planning, Research and Evaluation (OPRE) and Abt Associates under Martha R. Burt, Jill Khadduri, and Daniel Gubits. Data utilized was collected between 2010 and 2015; project was published in April 2016.

Subject(s): homelessness_prevention, temporary_housing, supportive_services

This brief explored the experience of sheltered homeless families, analyzing their access to public benefits and comparing them to other families in deep poverty in the same communities. It was concluded that sheltered homeless families access public benefits at rates equal to or greater than rates of those in deep poverty, thus implying that lacking the support of these programs does not appear to set families who become homeless apart from other deeply poor families who remain housed. Though this research does describe current efforts to connect families with poverty-level incomes to public benefits, they do not seem to address the root of the issue: preventing homelessness. Designing better programs and modifying current benefits / services may come more easily once the crisis or crises that lead families to lose their housing can be identified.

Behavioral Health Improvements Over Time among Adults in Families Experiencing Homelessness

Conducted with funding from HHS Office of Planning, Research and Evaluation (OPRE) and Abt Associates between late 2010 and early 2012 under Douglas Walton; Michelle Wood; Lauren Dunton.

Subject(s): mental_health, physical_health, emergency_housing

The researchers focused on parents' behavioral health due to their experiences with homelessness specifically analyzing psychological distress, alcohol dependence, drug abuse, and symptoms of post-traumatic stress disorder. They found that behavioral health problems experienced during homelessness may be temporary and helping families secure stable housing may be a crucial mental health intervention. They also found little evidence that psychological distress causes housing problems.

Child Separation among Families Experiencing Homelessness

Conducted with funding from HHS Office of Planning, Research and Evaluation (OPRE) and Abt Associates between late 2010 and early 2012 under Douglas Walton; Michelle Wood; Lauren Dunton.

Subject(s): temporary_housing, emergency_housing

The researchers found some correlation between housing instability and child separations as children who were separated three years after the stay in a shelter were more likely to be in families that had a prior history of homelessness before entering shelter and additional struggles with homelessness after the initial shelter stay.

Comorbid Health Conditions and Treatment Utilization Among Individuals with Opioid Use Disorder Experiencing Homelessness

Conducted with funding from HHS under Mir M. Ali; Harper Sutherland; and Emily Rosenoff

Subject(s): mental_health, substance_abuse

Opioid abusers experiencing homelessness present with higher rates of mental health conditions, e.g. bipolar disorder and schizophrenia, than those not experiencing homelessness. Those with opioid use disorder (OUD) also present with higher rates of alcohol and stimulant abuse than non-homeless individuals with OUD, which often coincides with lower rates of treatment for mental health conditions. The study concluded that underlying mental health conditions and substance abuse contributes toward adverse health outcomes in the homeless with OUD, and suggests that addressing housing instability might help those with OUD.

Establishing Eligibility for SSI for Chronically Homeless People

Conducted by the Office of Disability, Aging and Long-Term Care Policy (DALTCP) within the Office of the Assistant Secretary for Planning and Evaluation (ASPE) of the U.S. Department of Health and Human Services in partnership with Abt Associates under Martha R. Burt and Carol Wilkins from spring 2011 to its publication in May 2012.

Subject(s): supportive_services, temporary_housing

This brief helped to define Supplemental Security Income (SSI), explain its importance and elaborate on approaches to increase eligibility amongst homeless individuals. The paper describes how SSI provides additional income that people can contribute to rent, thus increasing their chances of becoming stably housed. This report also acknowledges particular programs and resources that offer approaches and techniques to help medical and other practitioners overcome some of the challenges involved with helping chronically homeless people qualify for SSI. However, there appears to be a lack of implementation of these programs by PSH tenants, indicating that efforts may want to focus on staffing SSI outreach and application assistance.

Evaluability Assessment of Discharge Planning and the Prevention of Homelessness

Conducted with The Assistant Secretary for Planning and Evaluation of the US Department of Health and Human Services under Moran, Garrett, Rafael Semansky, Elizabeth Quinn, Rebecca Noftsinger, and Teresa Koeni.

Subjects(s): homelessness_prevention

The study aims to determine the evaluability of discharge planning as a strategy to prevent homelessness. The researchers hope to answer if an evaluation of discharge planning in relation to the goal of homelessness prevention is feasible. Methods included reviewing current literature

on discharge planning, analyzing discharge planning related documents, and site visits to 8 of the 19 programs. The researchers found that a summative impact evaluation of discharge planning is not justified at this time since discharge planning is not easily distinguishable from other homelessness prevention programs. The discharge planning process is also not well-defined or consistently implemented across all the programs. Another key finding was that the best discharge planning cannot overcome a lack of affordable and suitable housing.

Exploring Homelessness Among People Living in Encampments and Associated Costs

Conducted with funding from HHS Office of the Assistant Secretary for Planning and Evaluation (ASPE) and HUD Office of Policy Development and Research (PD&R) in 2019, under Dunton, Lauren; Khadduri, Jill; Burnett, Kimberly; Fiore, Nichole; and Yetvin, Will of Abt Associates

Subject(s): unsheltered_homelessness

The leads conducted site visits to encampments in nine cities in order to collect information on expenditures and costs of encampments, and to observe encampments and interview residents. Findings include cost figures of between $3,393,000 and $8,557,000 annually on encampment related activities, which encompass outreach, clearance, prevention activities, and emergency shelter costs. Federal funding for encampments can be limited because funds are diverted from other areas, so cities are looking to alternative funding sources, such as special assessments and taxes. Lack of affordable housing is a primary causal factor in unsheltered living.

Final Report for the Independent Evaluation of the Substance Abuse Prevention and Treatment Block Grant Program

Conducted with funding from HHS through the Center for Substance Abuse Treatment and Center for Substance Abuse Prevention

Subject(s): substance_abuse

This research evaluates the effectiveness of the Substance Abuse Prevention and Treatment Block Grant (SAPT BG) Program, a nationwide funding effort to treat and prevent substance abuse. Evaluation shows that the program has clear measures of reducing substance abuse in implemented communities. Moreover, program implementation has pushed states to develop more expansive data collection and service infrastructure that has been beneficial at targeting the most vulnerable populations.

Health Conditions Among Individuals with a History of Homelessness Brief

Conducted with funding from HHS Offices of Planning and Evaluation and Behavioral Health, Disability, and Aging Policy between 2015 to 2019, under Harper Sutherland; Mir M. Ali; and Emily Rosenoff

Subject(s): physical_health, mental_health

The researchers analyzed a data set containing electronic health records (EHRs) of more than 54,000 individuals with a history of homelessness between 2015 and 2019 using the International Classification of Disease, Tenth Revision (ICD-10) code of homelessness to identify the group. They found that people with a history of homelessness are more likely to suffer from chronic conditions, hepatitis, and substance abuse, and have twice the rate of head injury.

Health, Housing, and Service Supports for Three Groups of People Experiencing Chronic Homelessness

Conducted with funding from the Office of Disability, Aging and Long-Term Care Policy (DALTCP) within the Office of the Assistant Secretary for Planing and Evaluation (ASPE) of the US Department of Health and Human Services in partnership with Abt Associates under Martha R. Burt and Carol Wilkins from October 2010 until its publication in February 2012.

Subject(s): supportive_services, mental_health, physical_health,

This study established an overview of the types of chronically homeless people who could benefit from Permanent Supportive Housing (PSH) and the types of care they are likely to need and receive as tenants. The types were split into three distinct categories: (1) Not Eligible for Medicaid, (2) Qualify for Medicaid, but not for specialized mental health services, (3) Qualify for Medicaid, and for specialized mental health services. Since each of these groups are composed of people with different needs, the brief specifically lays out what agencies and services would most adequately support those in each category. Another issue addressed throughout this study is the separation of physical and behavioral health care and the insufficient treatment that results due to a lack of information from either end. As a result, some PSH service-providers have started to integrate primary health care with mental health services, thus indicating why the separation of the three groups listed above is such a critical distinction.

Hispanic Families Experiencing Homelessness

Conducted with funding from HHS Office of Planning, Research and Evaluation (OPRE), and Abt Associates in 2014, under Jill Khadduri, Douglas Walton, Michael López, and Martha R. Burt.

Subject(s): emergency_shelter

The researchers analyzed 381 Hispanic families who entered emergency shelters and their experiences throughout the 20 months that followed. The study was exploratory, suggesting further research into the regional differences between Hispanic families and African Americans or white families is needed to better understand the observation that Hispanic families do somewhat better after staying in an emergency shelter.

Housing Assistance for Youth Who Have Aged Out of Foster Care: The Role of the Chafee Foster Care Independence Program

Conducted by US Department of Health and Human Services Office of the Assistant Secretary for Planning and Evaluation under Michael R. Pergamit, Marla McDaniel, Amelia Hawkins

Subject(s): youth_homelessness, supportive_services

The Chafee Foster Care Independence Program provides $140 million each year for independent living services to assist youths as they age out of foster care. This report focuses on how states spend their Chafee funding to provide housing support for people aging out of the system, especially states that indicated they would spend the maximum 30% on housing assistance — roughly ⅓ of all states. The methods used include analyzing published documents and web pages during the spring and summer of 2011 and contacting state Independent Living Coordinators, who administer the Chafee program in their home states. The researchers found that states typically use the housing funds to provide rental start-up costs, ongoing support, and emergency uses. Examples include covering security deposits, household goods, etc. A small number of states use Chafee funds in other ways, such as partnering with community organizations to help the youths. Some states have also developed their own programs, such as covering the costs of higher education. To allocate limited funding, most states limit support to youths in school or

working, with the hope that they will be able to pay rent after assistance ends. The study ends on a optimistic note with a mention of the Fostering Connections to Success and Increasing Adoptions Act of 2008, which allows states to use federal funds to keep young people in foster care until age 21.

Homeless Caseload is Associated with Behavioral Health and Case Management Staffing in Health Centers

Conducted with funding from the Bureau of Primary Care at the Health Resources and Services Administration (HRSA) of the U.S. Department of Health and Human Services under Emily B. Jones, Julia Zur, and Sara Rosenbaum. The data utilized was collected from the 2011 and 2012 Uniform Data System (UDS) – an administrative dataset reported to the Health Resources and Services Administration annually by all health center grantee, and the report was published in August 2015.

Subject(s): mental_health, continuum_of_care

This study aimed to answer whether there is an association between homeless caseload and the amount of resources that are allocated to homeless populations, ultimately finding that health centers with high homeless caseloads were more likely to have above-average behavioral health and enabling services staffing. These conclusions make logical sense, since a greater homeless rate will often lead to a greater need for behavioral health and enabling services, and luckily relative resource allocations reflect this demand. Some future research, however, may want to analyze how those who fall into the homeless population are able to afford these services. While Medicaid is one solution, not all states have expanded its reach and individuals may not have the insurance necessary to receive treatment or resources. In addition, community-based services at

health centers that do not always offer the full range of services needed by homeless patients, especially housing assistance. These are all future avenues for research.

Identifying and Serving LGBTQ Youth: Case Studies of Runaway and Homeless Youth Program Grantees

Conducted by the US Department of Health and Human Services Office of the Assistant Secretary for Planning and Evaluation (OPRE) in partnership with Mathematica Policy Research and The Williams Institute under Andrew Burwick, Vanessa Oddo, Laura Durso, Daniel Friend, and Gary Gates. The data utilized was collected from telephone calls and in-person visits/interviews conducted at four selected sites in spring 2013, and the report was published in February 2014.

Subject(s): youth_homelessness, supportive_services

This research project focused on LGBTQ Runaway and Homeless Youth (RHY) and aimed to better define this population whether it be by determining the size of these groups in local communities, the characteristics of particular subgroups (such as transgender youth and LGBTQ youth of color – both of which are considered high-risk subpopulations), particular risk and protective factors that are commonly encountered throughout the population, and the circumstances that ultimately contribute to their homelessness. The study was very broad but some of the key takeaways include that LGBTQ youth generally face risk factors similar to those of non-LGBTQ youth (although the frequency of these risks and circumstances contributing to them may differ for the two populations). In addition, some steps for serving LGBTQ RHY may be implemented more readily than others (i.e. prohibiting discrimination based on sexual orientation and gender identity may be relatively easy to implement, but establishing safer and

more welcoming environments can require more sustained effort in particular communities based on the existing stigma). This paper appears to have accumulated all of this information to create recommendations for policymakers and practitioners, rather than analyze their impact. As a result, future research may want to analyze how these recommendations have been taken into consideration, what initiatives have been implemented, and how LGBTQ RHY have since benefitted from these implementations.

Incarceration and Homelessness

Conducted with funding from HHS Office of the Assistant Secretary for Planning and Evaluation and HUD's Office of Policy, Development, and Research, U.S. Department of Housing and Urban Development under Stephen Metraux; Caterina Roman; and Richard Cho

Subject(s):

This brief reviews the connections between incarceration and homelessness in the US. First, it presents studies that investigate whether incarceration causes homelessness, or vice versa. These studies reveal that many incarcerated individuals become homeless and that many of those experiencing homelessness individuals have been incarcerated. However, most data is correlational rather than causational. The brief analyzes the barriers to accessing housing for formerly incarcerated people as well as potential solutions to the issue. Finally, researchers describe how frequently former incarcerees engage in different types of housing assistance along a continuum from permanent independent housing to institutional shelters.

Individuals Experiencing Homelessness are Likely to have Medical Conditions Associated with Severe Illness from COVID-19

Conducted with funding from HHS Office of the Assistant Secretary For Planning And Evaluation and Office of Disability, Aging, and Long-Term Care Policy between 2015 and 2019, under Harper Sutherland; Mir M. Ali; and Emily Rosenoff.

Subject(s): physical_health, mental_health

The researchers used IBM Watson Health's Explorys all-payer EHR dataset to look at the prevalence of medical conditions associated with severe health problems from COVID-19 among homeless populations. The CDC listed that people with asthma, liver disease, chronic lung disease, diabetes, and other respiratory illnesses are at a higher risk for more severe illness from COVID-19, and the homeless cohort is at a higher risk for contracting COVID-19 and for having these health conditions.

Lessons from the Field: Using Housing Vouchers to Support Youth with Child Welfare Experience at Risk of Homelessness

Conducted with funding from HHS under Kelsey Chesnut, Megan Shoji, and Morgan Woods

Subject(s): supportive_services, homelessness_prevention, youth_homelessness

The brief explores housing voucher programs within California and New Jersey, advocating for housing vouchers as a means to prevent homelessness and instability in high-risk youth. The main recommendations include supporting youth with resources and information in order to access vouchers, and for organizations to continue communication with both youth and landlords to ensure youth stay housed and encourage landlords to accept high-risk tenants that they would normally be disincentivized to accept.

Leveraging Data to Support Children Experiencing Homelessness

Conducted with funding from HHS during January through February 2018 under Sara Shaw, Ashley Hirilall, and Tamara Halle

Subject(s): youth_homelessness, supportive_services

There are several challenges to capturing an accurate count of the amount of children experiencing homelessness due to definitional differences of homelessness between departments, limited methods of data collection, the high mobility often accompanying homelessness, and self-reporting stigma. As a result, there are many gaps in the accounting of young children (ages 0-5) who are the most vulnerable age group to experience homelessness yet one of the most neglected in research. By conducting interviews with key specialists in the early child homelessness field researchers found that there were several ways agencies could improve their tracking of homeless children, including increasing data sharing between agencies, asking for housing status more frequently (for example, in subsidy recipients), and including young children in more longitudinal data on homelessness. This report indicates that data recording is an important prerequisite for programs to be able to successfully access their target populations. Moreover, it demonstrates that data collection on homelessness is very decentralized, and methods differ drastically from state to state, which presents a serious challenge.

Linking Human Services and Housing Assistance for Homeless Families and Families at Risk of Homelessness

Conducted with funding from the Office of Planning, Research and Evaluation (OPRE) Administration for Children and Families of the U.S. Department of Health and Human Services in partnership with Abt Associates under Alvaro Cortes, Lauren Dunton, Meghan Henry,

Howard Rolston, and Jill Khadduri. The research period was September 2010 to August 2011 and was published in April 2012.

Subject(s): supportive_services

This study aimed to find the programs that exist nationwide that deliberately integrate human services and housing support for homeless families. The ultimate goal was to utilize the knowledge gained from these existing sites to create promising practices that facilitate the development, implementation, and sustainability of these programs. Some of these practices include tying overall objectives to their target population, service interventions, and partnership arrangements, resulting in more focused programs. In addition, there appeared to be a large emphasis on recurring themes such as fostering strong partnerships and reliable relationships whether it be with helpful organizations, case managers, housing specialists, or the landlords themselves. Ultimately strengthening this relationships, optimizing necessary funding, and developing clear missions will help to more efficiently implement programs.

Medicaid and Permanent Supportive Housing for Chronically Homeless Individuals: Literature Synthesis and Environmental Scan

Conducted under Martha R. Burt, Carol Wilkins and Danna Mauch

Subject(s): supportive_services

The researchers build upon the conclusion that permanent supportive housing (PSH) as a resource for the homeless is one of the most effective solutions in maintaining health, offsetting welfare costs, and stabilizing consistency of housing. The report finds that the federal government may not be utilizing some of its most cost avoidant strategies for homelessness in

the long run. It also highlights the lack of support and obstacles for PSH to manifest into reality and cites specific health benefits as a result of PSH that would be useful for further analysis.

Medicaid and Permanent Supportive Housing for Chronically Homeless Individuals: Emerging Practices From the Field

Conducted with funding from the Office of Disability, Aging and Long-Term Care Policy (DALTCP) within the Office of the Assistant Secretary for Planning and Evaluation (ASPE), U.S. Department of Health and Human Services in partnership with Abt Associates. This project collected data from October 2010 to 2013 and was published in August 2014.

Subject(s): physical_health, mental_health, substance_abuse

This study specifically focused on identifying how states and communities are using and modifying Medicaid programs in order to improve healthcare systems and better serve people with histories of chronic homelessness. The brief highlights how Medicaid programs are successful in offering coverage opportunities for a variety of necessary services to PSH individuals before and even after they move into housing. It also explains how the Affordable Care Act in 2014 has only helped to expand these opportunities.

Mental Health of Vulnerable Youth and their Transition to Adulthood: Examining the Role of the Child Welfare, Juvenile Justice, and Runaway/Homeless Systems

Conducted with funding from Office of the Assistant Secretary for Planning and Evaluation (ASPE) between 1994-1995 and 2001-2002 under Elizabeth C. Hair, Ph.D., Kathleen Sidorowicz, Laurie Martin, and Alyssa Milot of Child Trends

Subject(s): mental_health, youth_homelessness

The brief analyzed the mental health of vulnerable youths who have been in contact with social service systems —defined as child welfare services, juvenile justice, and homeless shelters. Key research areas include the percentage of youth who had contact with multiple service systems, how youths who had contact with service systems differed from youths who did not, and if contact with service systems increases the likelihood of receiving mental health services.

By analyzing data in the National Longitudinal Study of Adolescent Health, the study found that around 10% of youths had contact with at least one service system. Youths who had contact with service systems are more likely to fall into the Troubled Youth profile, the Alcohol Interference youth profile, the Depressed youth profile, and the Depressed and Delinquent Plus youth profile than their no service system contact counterparts. Youths with worse mental health as they transition to adulthood faced more difficulty in young adult experiences and are more likely to receive public assistance. Finally, youths who had any service system contact were more likely to require counseling services.

Potential Analyses with Homelessness Data: Ideas for Policymakers and Researchers

Conducted by HHS under Michelle Wood and Jill Khadduri

Subject(s): supportive_services

This report primarily outlines gaps for states to allocate data of welfare services (TANF and Medicaid) to better study their interaction and effectiveness on homeless populations. The researchers outline several ways state governments could further investigate how homelessness may be prohibitive to service access, how effective services are at preventing homelessness, and whether they are underused by homeless populations. The report provides helpful guidelines to

state legislators for how to collect data in the future to streamline into more research on homelessness.

Predicting Repeated and Persistent Family Homelessness: Do Families' Characteristics and Experiences Matter

Conducted by HHS during late 2010- early 2012 under Zachary Glendening and Marybeth Shinn

Subject(s): youth_homelessness, emergency_housing

Findings show that using family history/background/characteristics as explanatory variables for rates of repeated homelessness has a negligible effect. However, child separation from their family and parental employment are two factors that appear in both measures of repeated homelessness. Additionally, when looking at characteristics of parents specifically, two factors correlated with repeated homelessness rates: having parents who have experienced homelessness as an adult or as a child. One of the uses of this study is to disprove the need for a biased system to determine which people in emergency shelters get housing first, given that behavioral background, drug history, family background, etc, is shown to have little impact on whether families will successfully "escape" homelessness. The study also suggests that the risk factor being parents' previous homelessness means we should look into trauma-healing care to mitigate the effects of previous experiences of homelessness on repeated counts. Author's note that most family units were single-adult (usually single mothers), and this could be a point of further research.

Pretesting a Human Trafficking Screening Tool in the Child Welfare and Runaway and Homeless Youth Systems

Conducted with funding from HHS, between March 2016 and November 2016, under the Urban Institute

Subject(s): youth_homelessness, supportive_services

By creating and pretesting the Human Trafficking Research Tool (HTST), the Urban Institute has developed a 19-item tool and a 6-item short version (HTST-SF) that identified trafficking victims 61 percent of the time and non victims 84 percent of the time, based on its agreement with practitioners' assessments of youth's trafficking experiences in New York, Texas, and Wisconsin. HTST responses were significantly correlated with several risk factors and outcomes associated with trafficking victimization, including running away from home, being kicked out by parents or guardians, exchanging sex on their own for something of value, and abusing over-the-counter drugs. Practitioners gave positive feedback on the Life Experiences Survey and its ease of administration, and agreed that youth understood the HTST questions and gave truthful responses. The HTST was also accessible to the targeted youth population: items were written at a 6th-grade level and youth spent only about two minutes completing the HTST and less than a minute completing the HTST-SF. Based on these findings, we conclude that the HTST and HTST-SF is an accessible, easy-to-use set of screening tools for identifying youth who are victims of human trafficking in child welfare and runaway and homeless youth settings.

Promising Practices for Children Experiencing Homelessness: A Look at Two States

Conducted with funding from the US Interagency Council on Homelessness (USICH), published May 2014, under HHS Office of the Deputy Assistant Secretary for Early Childhood Development

Subject(s): youth_homelessness, physical_health, mental_health

Children aged 0-5 who are experiencing homelessness are particularly vulnerable to a host of negative outcomes, including behavioral and developmental delays, physical disabilities, and social-emotional issues. Increasing access to high-quality early care and learning programs for this population will contribute to their healthy development and resiliency. Federal programs including Head Start and Early Head Start, Child Care and Development Fund, Early Childhood State Advisory Councils, McKinney-Vento Homeless Education programs, and the Race to the Top-Early Learning challenge have given states and localities opportunities to tackle this problem. For instance, Massachusetts and Oregon leveraged their Race to the Top - Early Learning Challenge grants (RTT-ELC) to make their state's early learning system more accessible to children experiencing homelessness. Lessons learned from Massachusetts and Oregon provide other states with an opportunity to develop targeted interventions for young children experiencing homelessness.

Public Housing Agencies and Permanent Supportive Housing for Chronically Homeless People

Subject(s): supportive_services

Conducted by HHS under Carol Wilkins and Martha R. Burt

Chicago and Oakland provide case studies of flagship programs in providing permanent housing solution (PSH) as a permanent housing solution in their areas. However, there are challenges in

collaboration with Public Housing Agencies (PHA's). The case studies have used their moving to work (MTW) status to implement programs, however even non-MTW PHA's have capabilities to implement PSH. The research speaks to how a permanent housing solution without end dates or time limits — as well as comprehensive resources — might provide more security for formerly homeless people with disabilities. In addition, it explores how organizations can implement PSH despite having MTW status or not.

State Strategies for Coordinating Medicaid Services and Housing for Adults with Behavioral Health Conditions

Conducted with funding from HHS' Office of the Assistant Secretary for Planning and Evaluation (ASPE) in partnership with Mathematica Policy Research under Rebecca Kleinman, Matthew Kehn, Allison Wishon Siegwarth, and Jonathan Brown. The data utilized was collected from a review of documents and on telephone / in-person interviews conducted in fall 2014 and published in April 2015.

Subject(s): mental_health, physical_health, supportive_services, substance_abuse

This brief looks at the current strategies of four states (Louisiana, Massachusetts, Tennessee, and Illinois) that have been implemented to improve the link between Medicaid and housing services for adult Medicaid beneficiaries with behavioral health conditions. For a brief summary of each states' strategies: Louisiana's current Permanent Supportive Housing (PSH) program offers Medicaid-covered behavioral health and long term care services, Massachusetts provides community-based care coordination for chronically homeless adults in supportive housing, Tennessee has implemented a statewide, mandatory managed care program serving the entire state's Medicaid population, and Illinois has established partnerships of community-based

providers that include behavioral health, physical health, housing, and social service agencies. The study ultimately concludes that Medicaid does provide individual states with the flexibility to invest in initiatives to improve the coordination of behavioral health and housing services. All four of these states and the strategies they have implemented are different from one another, but they ultimately illustrate how different financing authorities, reimbursement mechanisms, and partnership structures can all still achieve the same goal. Since this paper was published in 2015, future research projects may want to analyze how other states have learned from these models and what new strategies have been implemented to further develop these initiatives.

State Strategies for Improving Provider Collaboration and Care Coordination for Medicaid Beneficiaries with Behavioral Health Conditions

Conducted with funding from HHS' Office of the Assistant Secretary for Planning and Evaluation (ASPE) in partnership with Mathematica Policy Research under Rebecca Kleinman, Matthew Kehn, Allison Wishon Siegwarth, and Jonathan Brown. The data utilized was collected from a review of documents and on telephone / in-person interviews conducted in fall 2014 and published in February 2015.

Subject(s): mental_health, physical_health, unsheltered_homelessness, supportive_services

This brief looks at the current efforts of four states (Louisiana, Massachusetts, Tennessee, and Illinois) that have been made to facilitate provider-level coordination for Medicaid beneficiaries with behavioral health disorders. For a brief summary of each states' strategies: Louisiana contracted with a single statewide managed behavioral health organization (MBHO) to manage all behavioral health services for Medicaid and non-Medicaid adults (as part of a larger effort

called the Louisiana Behavioral Health Partnership (LBHP) to improve the accessibility and outcomes of behavioral health services), Massachusetts implemented the Community Support Program for Ending Chronic Homelessness which provides Medicaid reimbursement for community-based coordination support to individuals in PSH, Tennessee includes behavioral health and long-term care services and supports in its managed care contracts, and Illinois has established partnerships of community-based providers that include behavioral health, physical health, housing, and social service agencies. The study ultimately concludes that Medicaid does provide individual states with the flexibility to invest in initiatives to improve the coordination of behavioral health and housing services. All four of these states and the efforts they have implemented are different from one another, but they ultimately illustrate how different financing authorities, reimbursement mechanisms, and partnership structures can all still achieve the same goal. Since this paper was published in 2015, future research projects may want to analyze how other states have learned from these models and what new strategies have been implemented to further develop these initiatives.

Strategic Action Plan on Homelessness

Conducted by HHS through Secretary's Work Group on Ending Chronic Homelessness

Subject(s): homelessness_prevention

The plan emphasizes several key updates to the ways in which HHS will further strategize to tackle homelessness, including a broader consideration of family members, collaboration with other federal agencies, addressing homelessness in the context of a disaster, and emphasizing preventing first-time homelessness. The report outlines strategies for state governments to more effectively respond to and prevent chronic homelessness. In addition, it compiles the funding and

results of key research and programming activities regarding homelessness from 2003-2006. The report is an important guide for executive offices to implement proper data collection and initiatives to respond to homelessness in their communities.

Study of HHS Programs Serving Human Trafficking Victims

Conducted by HHS under Heather J. Clawson, Nicole M. Dutch, Amy Salomon. Lisa Goldblatt Grace

Subject(s): supportive_services

The researchers discuss gaps in connecting victims of trafficking to benefits and resources, including housing. There are challenges to identifying trafficking victims themselves, both international and domestic. While federal funding is available to provide resources, language barriers and transportation serve as two of the biggest obstacles in victims accessing services. The research warrants more need for study into not just international trafficking victims, but also domestic ones, and how to better strategize care access. Trafficking victims are a unique subset of those with housing instability, because they have additional needs and conditions as well, including trauma, and guardianship. This report, like others, emphasizes the power of working with community members to make more informed decisions.

The Implementation of Maternity Group Home Programs: Serving Pregnant and Parenting Teens in a Residential Setting

Contracted by the Office of the Assistant Secretary for Planning and Evaluation (ASPE) under Lara K. Hulsey, Robert G. Wood, Anu Rangarajan (Mathematica Policy Research)

Subject(s): youth_homelessness

This study documents the implementation of maternity group home programs. The researchers analyzed maternity group home programs in 7 states: Georgia Campaign for Adolescent Pregnancy Prevention Second Chance Homes, St. Andre Group Homes in Maine, Teen Living Program in Massachusetts, Teen Parent Supportive Housing Services Collaborative in Michigan, Teen Parent Program in New Mexico, Inwood House Maternity Residence in New York, and Friends of Youth Transitional Living Program in Washington. The researchers analyzed management, funding, target population, services provided, and staffing at each program. The report found that maternity group homes are often part of a larger organization that provide broader social services. The programs in the study rely on various funding sources, including federal funding, state funding, donations, and even contributions from people living in the homes. Many residents in the maternity group homes reported experiencing abuse and coming from unstable family situations. There are often eligibility requirements the residents must meet to reside in the homes, which may include fitting the HUD definition of homelessness. All of the maternity group homes visited provide housing, supervision and structure, and case management.

The 1996 National Survey of Homeless Assistance Providers and Clients: A Comparison of Faith-Based and Secular Non-Profit Programs

Conducted by the Office of the Assistant Secretary for Planning and Evaluation and US Department of Health and Human Services with a 1996 survey (study published in 2002) under Laudan Y. Aron and Patrick T. Sharkey

Subject(s): supportive_services

This study uses the data from the National Survey of Homeless Assistance Providers and Clients to determine the role that faith-based homeless services play in homeless assistance by

comparing faith-based and secular non-profit service agencies. The study found that faith-based organizations run about a third of all homeless programs, including the majority of food programs. They are also more prominent in urban areas and in the southern region of the country. Faith-based providers serve a more diverse group of people than secular providers, such as single men, single women, women with children, families with children, and youths. Furthermore, faith-based organizations are less likely than secular organizations to have a special focus, such as only supporting domestic abuse victims. Lastly, faith-based organizations receive substantially less government funding than secular organizations, with 22% of secular non-profits relying exclusively on government funds compared to less than 3% of faith-based groups. Furthermore, 62% of faith-based programs receive no government funding compared to less than 25% of secular groups.

Well-being of Young Children after Experiencing Homelessness

Conducted with funding from HHS Office of Planning, Research and Evaluation (OPRE), and Abt Associates between late 2010 and early 2012, under Scott R. Brown; Marybeth Shinn, and Jill Khadduri.

Subject(s): youth_homelessness, emergency_housing

The researchers looked at a data set of 925 children of homeless families from ages 59 months (less than 5 years old) to 18 after they stayed in an emergency shelter. The researchers then compared the children's pre-reading skills, pre-math skills, and behavior problems to the national norm. They found that compared to national norms, children who have stayed in shelters have moderately weaker pre-reading skills, slightly weaker pre-math skills, higher rates of behavioral problems, and a higher risk for development delays. The researchers also found that enrollment

in early education programs like Head Start correlates with stronger pre-math and pre-reading skills, but enrollment in these programs is lower for families experiencing housing instability.

Summary of Findings

Literature compiled from the Department of Health and Human Services emphasized the tangible effect of housing insecurity on human health while focusing on the effectiveness of various interventions and health-based programs. The literature highlights that homelessness is a serious public health concern for children and adults alike.

In terms of COVID-19, homelessness has been found to put one at a higher risk for more severe illness from COVID exposure because of underlying conditions prevalent among those experiencing homelessness (diabetes, chronic lung disease, COPD, asthma, and other respiratory illnesses). Multiple studies were devoted to adolescent well-being after experiencing homelessness or shelter stays, and interestingly, the researchers used academic performance (reading, writing, mathematics skills, etc.) and classroom behavior as a metric of well-being. Several studies also examined the effectiveness of specific services, such as Medicaid, permanent supportive housing (PSH) and public public housing agencies (PHA), and services specific to youth like the child welfare and juvenile justice systems. The researchers were also concerned with eligibility for social supportive services like PSH, Medicaid, and supplemental security income (SSI) and making sure that those services were available to individuals before and after securing housing.

Department of Housing and Urban Development

Literature

Addressing Housing Affordability in High-Cost Metropolitan Areas in the United States

Conducted with funding from HUD from 2018

Subject(s): affordable_housing

In investigating new housing starts in high-cost cities, research leads found that a lack of affordable housing is "a reality in high-cost metropolitan areas across the country." The report states that of all the regions in the US, the West — particularly metropolitan areas on the West Coast, mostly in California — is where low-income groups struggle the most to access affordable housing. The report identifies which metropolitan areas have "the greatest affordability challenges," and recommends solutions for these communities, such as offering technical assistance to local governments, facilitating development of more rental housing through financing tools, and using vacant properties for new affordable housing units.

American Housing Survey: Housing Adequacy and Quality As Measured by the AHS

Conducted by Frederick J. Eggers and Fouad Moumen from Econometrica, Inc. in March 2013 for HUD's Office of Policy Development and Research

Subject(s): housing_market

The American Housing survey is a measure of the physical adequacy of a housing unit, which is measured by asking questions about running water, bathroom facilities, indoor plumbing, heating equipment, electricity and electrical safety, structural stability, the presence of rats, peeling paint, etc. The study found that the most frequently observed problems were bathrooms shared between

units (55 percent) and heating failure (29 percent). Other problems included a lack of cold and hot water, a shower or tub, or a flush toilet.

A Picture of Disability and Designated Housing

Conducted with funding from Economic Systems Inc. under Casey J. Dawkins, Ph.D., and Mark Miller of the National Center for Smart Growth on March 6, 2015

Subject(s): physical_health, mental_health

This report examined the "characteristics of federally-assisted housing designated for disabled households living in the U.S. Prior research suggests that disabled persons face significant challenges when trying to secure affordable, high-quality housing;" this study asked whether HUD housing was adequate enough to address the growing needs of the baby boomer population as it aged and became more likely to have a disability. The report gave an overview of major HUD programs designed to serve the needs of the disabled population, and a description of the data used for the analysis and research methods of the report. The report found that disabled persons were most prevalent in the tenant-based voucher programs, but less than five percent of households receiving such assistance received it from programs directly from programs targeted toward the disabled. The finding pointed to a need for more community outreach to identify potentially eligible and interested disabled households of HUDs disability assistance program.

Assessment Tools for Allocating Homelessness Assistance: State of the Evidence

Conducted with funding from HUD in November 2014 under the Office of Policy Development and Research

Subject(s): supportive_services

"On November 5, 2014, HUD's Office of Policy Development and Research, in partnership with the National Alliance to End Homelessness (the Alliance), convened a panel of experts to discuss assessment tools that communities are using to allocate homeless assistance and to consider the evidence base for the questions used in the tools." Experts agreed that "existing assessment tools do not have a strong evidence base and are limited in their ability to select the best interventions for families and individuals or to predict which families would be the most successful in different interventions." They called for additional research and other approaches such as using a lottery or providing the "lightest touch" to everyone to help as many people as possible and determine who needs additional help.

A Qualitative Assessment of Parental Preschool Choices and Challenges Among Families Experiencing Homelessness

Conducted with funding from HUD between 2016 and 2020, under Lindsey Stillman, Kate Hurd, Charles Kieffer, Jamie Taylor, Britton Gibson from The Cloudburst Group

Subject(s): youth_homelessness

This research study aimed at understanding the challenges and decision making process that influenced homeless parents to send their children or not to preschool, a crucial decision in future childhood success. The research showed that "key factors influencing preschool enrollment are housing stability, access to social-support networks, parental response to early learning environments, and the types of facilitative support for preschool enrollment received during interactions with early childhood and social service systems." From these factors, the researchers established a socioecological conceptual framework and called for a variety of solutions including increasing collaborative partnerships between homeless services providers and

educators, incorporating discussion of preschool options and mechanisms for accessing preschool as part of a family's housing crisis services in homeless shelters, and clarifying guidance and providing encouragement for school staff acting as homeless liaisons.

Characteristics of HUD-Assisted Renters and Their Units in 2003

Conducted with funding from HUD's Office of Policy Development and Research (PD&R) during May 2008 by US Department of Housing and Urban Development, Housing and Demographics Analysis Division Office of Policy Development and Research

Subject(s): rent_assistance

This report compares HUD-assisted households with all income-eligible renters using data from the American Housing Survey. The study found that in 2003, 49% of renter households were eligible for housing assistance under HUD. Only 26% of income-eligible renters reside in HUD-assisted rental units. Additionally, compared with all income-eligible renters, those in assisted households are more likely to be minorities, be older, and have lower levels of education. Assisted households are more likely to be female-led households, smaller in size, have lower incomes, pay lower rents, and have lower rent-to-income ratios than all income-eligible renters.

Chicago, Illinois Community Encampments Report

Conducted with funding from HUD in fall 2019

Subject(s): temporary_housing

Researchers visited four communities to "collect information about each community's response to encampments and the costs associated with those responses." This particular study focused on

Chicago, Illinois. In the years leading up to the study, encampment size and visibility had increased despite homelessness going down between 2017 and 2019. Much of the study focuses on characterizing the encampments and those living in them, of which 70 percent were Black/African American and 70 percent were men mostly between the ages of 35 and 65. 80 percent of encampment residents did not have relatives or partners with them; there were no families with children and very few veterans. The study details interactions between public and private agencies like the Chicago Department of Law and the Salvation Army with the population living in encampments.

Child and Partner Transitions among Families Experiencing Homelessness

Conducted with funding from HUD Offices of Planning, Research, and Evaluation (OPRE) and Assistant Secretary for Planning and Evaluation (ASPE), between 2010 and 2015, under Lauren Dunton; Douglas Walton; Lincoln Groves of Abt Associates

Subject(s): emergency_housing, youth_homelessness

This analysis illuminates the family dynamics of those who experience homelessness in emergency shelters. Families experienced significant instability in their composition, i.e. reuniting with partners/family members or new separations. While formal separation of children at shelter entry was not common, additional families reported placement of children in foster care 20 months later. Administrators of programs that serve homeless families should recognize that there are a significant number of informal child and partner separations and consider whether their current policies allow all family members to stay with the family. Emergency shelters that receive HUD funding are now required to permit adolescent males and male heads of households to stay with their families.

Choice Matters: Housing Models that May Promote Recovery for Individuals and Families Facing Opioid Use Disorder

Conducted with funding from HUD Office of the Assistant Secretary for Planning and Evaluation (ASPE), published June 2019, under Abt Associates

Subject(s): mental_health, supportive_services, Housing_First

Although MAT has been well documented to assist in recovery from OUD, individuals experiencing homelessness can have more difficulty adhering to MAT than individuals in stable housing. Opioid overdose is more common in individuals who experience homelessness than in individuals who inject drugs but are not homeless. Discussions with experts and program staff emphasize that it is crucial to support individuals and families in choosing the housing and services that work best for them at each stage of recovery and individuals may want to move between models at different points in time. The models of Housing First, PSH, and recovery housing all show promise when the individuals experiencing OUD have chosen that type of housing.

Consumer Integration and Self-Determination in Homelessness Research, Policy, Planning, and Services

Conducted with funding from Office of the Assistant Secretary for Planning and Evaluation, U.S. Department of Health and Human Services; and the Office of Policy, Development, and Research, U.S. Department of Housing and Urban Development during September 2007 under Susan Barrow, Lorraine McMullin, Julia Tripp, Sam Tsemberis

Subject(s): supportive_services

This study looks at how the process and outcomes of research, policy, and service delivery change with the involvement of people who have personally experienced homelessness. They found that involving individuals who have experienced homelessness adds to the relevance, validity, and sensitivity of homelessness research, service planning, and policy-making, while also benefiting the involved individual through personal growth and improved self-confidence.

Continuity and Change: Demographic, Socioeconomic, and Housing Conditions of American Indians and Alaska Natives

Conducted by HUD's Office of Policy Development and Research (PD&R) in January 2014 under Kathryn L.S. Pettit, G. Thomas Kingsley, Jennifer Biess, Kassie Bertumen, Nancy Pindus, Chris Narducci, and Amos Budde

Subject(s): housing_market, supportive_services

This report characterized the demographic, socioeconomic, and housing conditions of indigenous Americans, referred to in the report as "American Indians and Alaska Natives." The indigenous American population increased by 18% from 2000 to 2010, double the national growth average. Homeownership of studied households lagged behind the national average, though the rate of owner-occupied housing united rose twice as fast compared to the national average across the time period. The conclusion found that in the studied areas, the Native American Housing Assistance and Self-Determination Act was the dominant framework for the delivery of housing assistance, that population growth fuels housing needs, and that gaps in well-being between the focus population and the national average are sizeable.

Developing Choice Neighborhoods: An Early Look at Implementation in Five Sites - Interim Report

Conducted by The Urban Institute for HUD's Office of Policy Development and Research in September 2013.

Subject(s): housing_market, supportive_services, affordable_housing

This report describes the early results of implementing the Choice Neighborhoods program (Choice), whose goal is to "redevelop distressed assisted housing programs and transform the neighborhoods surrounding them into mixed-income high-opportunity housing." The five cities that were selected for the study were Boston, Chicago, New Orleans, San Francisco, and Seattle. It concluded that a number of strategies could be successful under the notion that different neighborhoods require different approaches. These can range from moderate change to complete overhauls of the entire neighborhood.

Domestic Violence Housing First (DVHF) Demonstration Evaluation

Conducted with funding from Washington State Coalition Against Domestic Violence, contracted in 2022 by the Office of the Assistant Secretary for Planning and Evaluation (ASPE), under Michigan State University

Subject(s): Housing_First, supportive_services

This report presents the impacts of the DVHF model on domestic violence survivors and their children over twenty-four months. Longitudinal evidence from this demonstration evaluation indicates that the DVHF model is more effective than SAU in helping DV survivors obtain and maintain safe and stable housing over time. The researchers found that while the "services as usual" that DV agencies provide may positively impact survivors' safety and well-being

(Sullivan, 2018), providing mobile advocacy and flexible funding appears to be especially salient in achieving stable housing.

Employment of Families Experiencing Homelessness

Conducted with funding from HUD, over a 37 month period from 2010 to 2015, under Lauren Dunton of the Office of Planning, Research, and Evaluation (OPRE) and Abt Associates

Subject(s): supportive_services

Many families who experience homelessness or deep poverty have weak connections to the workforce. 2,282 families (including over 5,000 children) enrolled in the Family Options Study were followed over the project period. Although their rate of employment rises three years after entering shelter, homeless families still have a lower employment rate than those whose housing is relatively stable or who do not enter emergency shelters. At the time they were in shelter, parents reported difficulty finding jobs and barriers to work that included child care and other family responsibilities. Families experiencing homelessness face multiple barriers to employment and childcare and family responsibilities. Programs and funding streams such as TANF, the Workforce Innovation and Opportunity Act (WIOA), and the Supplemental Nutrition Assistance Program Employment and Training Programs (SNAP E&T) may support parents in overcoming barriers to seeking and maintaining employment, which may include subsidized child care or links to employment through referrals, job search assistance, and skills training. When developing strategies for ending family homelessness, a community should look to connect families with employment services providers that follow evidence based models and have strong track records of helping families find and retain employment.

End of Participation in Assisted Housing: What Can We Learn About Aging in Place?

Conducted with data from 2000—2008

Subject(s): supportive_services

This study seeks to identify the viability of an alternative to assisted housing — "aging in place." The study characterized and quantified those in assisted housing, particularly the neighborhood and its demographics, and the types of units occupied. Among its key findings, the report found that the average age at the end of participation (EOP) was 78, but higher among some program types (79 for the multifamily program) and lower for others (76 for vouchers). It also found that 27 percent of elderly households stayed on assistance until they were at least 85 years of age and that households made up of a single individual were more likely to stay until an advanced age. Households headed by a woman, persons without disabilities, and those who identified as white were more likely to stay to more advanced ages (by two to four years) than households headed by those who did not fit these categories.

Evaluation of the HUD-DOJ Pay for Success Permanent Supportive Housing Demonstration Baseline Report

Conducted with funding from HUD and DOJ between 2019 and 2020, by Kelly Walsh, Nicole DuBois, Samantha Batko, Clare Salerno, Eleanor Noble, Akiva Liberman, Mary Cunningham

Subject(s): supportive_services

In 2016, HUD and DOJ signed an interagency agreement and launched the Pay for Success Permanent Supportive Housing Demonstration, which aimed to "strengthen communities' ability to reduce recidivism and homelessness among the reentry population by increasing Permanent Supportive Housing (PSH) through Pay for Success (PFS), a promising financing mechanism

that unlocks private and philanthropic investment in social programs". Seven sites received $8.7 million in funding and this report aimed to evaluate the effectiveness of these program implementations. Researchers noted that the program has had initial success despite challenges like the time it takes to engage partners and structure a PFS contract. Sites reported "increased attention to the target population and the efficacy of PSH, new and strengthened partnerships, and improved capacity in the jurisdiction". Researchers aimed to continue researching the program in future research.

Evaluation of the HUD Youth Homelessness Demonstration Program

Conducted with $33 million in funding from HUD, between January 2017 to 2021, under Westat's Debra J. Rog; Kathryn A. Henderson; and M. Kerr

Subject(s): youth_homelessness, Continuum_of_Care

The baseline survey indicates that most youth homeless service systems are in the early or medium stages of development and face many challenges serving youth and young adults at risk of or experiencing homelessness; less than 25 percent of the communities report having highly developed systems at the time of the survey. The following are the main limitations the Continuums of Care identified: (1) lack of available services, providers, or research to serve 14-17-year-olds; (2) limited affordable housing or educational and employment opportunities; (3) limited coordination across agencies like the healthcare system and juvenile justice; (4) difficulties serving minors due to regulatory or policy restrictions that prevent minors from "signing leases, consenting to data sharing, and receiving shelter or services without parental consent"; (5) a lack of data on youth at risk of or experiencing homelessness.

Family Options Study: 3-Year Impacts of Housing and Services Interventions for Homeless Families

Conducted from September 2010–January 2012

Subject(s): rapid_rehousing

The study began after "2,282 families enrolled in the Family Options Study across 12 communities after spending at least 7 days in emergency shelter." Families were assigned to four groups: "(1) SUB, in which families have priority access to a permanent housing subsidy; (2) CBRR, in which families have priority access to community-based rapid re-housing; (3) PBTH, in which families have priority access to project-based transitional housing; or (4) UC, in which families have access to usual care homeless and housing assistance but do not have priority access to any particular program." This report is the three-year impact study of three interventions in five domains related to family well-being: housing stability, family preservation, adult well-being, child well-being, and self-sufficiency. Families given priority access to assistance obtained housing subsidies sooner. The study also found that those in the CBRR group did about as well as families assigned to the UC group, but at a lower cost. The conclusions suggest that families experiencing homelessness can successfully use and retain housing vouchers that give them priority access to permanent housing studies that have many benefits at some additional cost.

Family Options 12-Year Study

Conducted with funding from HUD

Subject(s): rent_assistance, rapid_rehousing, temporary_housing

The Family Options Study compares the effectiveness of 1) a long-term rent subsidy, 2) a short-term rent subsidy (rapid rehousing), and 3) project-based transitional housing. The study formed interesting relationships between the specific treatment a person receives and their future choice of support. For example, people who were provided traditional housing and shelter support tended to lean towards HCV (Home Choice Voting), while the other interventions resulted in more range throughout votes. Furthermore, families that were offered long-term rent subsidies experienced reductions in homelessness and residential instability relative to the usual care. The usual care provided to families costs about $41,000 per family over 37 months — the gap between the usual and long-term subsidies is about 9%. That extended funding played critical roles in addressing food insecurity, family separation, and psychological distress — making the investment much more significant beyond finances.

Family Unification Program - Family Self-Sufficiency (FUP-FSS) Demonstration Evaluation

Conducted with funding from HUD between 2016 and 2020, under Claudia Solari and other authors from the Urban Institute.

Subject (s): youth_homelessness

This research project aimed at understanding the effectiveness of the Family Unification Program - Family Self-Sufficiency (FUP-FSS) demonstration, undertaken since the summer of 2016, to fight the increasing phenomenon of youth aging from foster care into homelessness. This solution consists of combining housing choice vouchers for eligible youth under the Family Unification Program (FUP) with financial empowerment services under the Family Self Sufficiency (FSS) program. The study showed that FUP-FSS had advantages and disadvantages.

The limited number of youth interviewed valued the FSS services and support they received, yet, the researchers encountered implementation issues like low overall demonstration participation and low levels of collaboration among public housing agencies and public child welfare agencies.

Homelessness Prevention Study - Prevention Programs Funded by the Homelessness Prevention and Rapid Re-Housing Program

Conducted with funding from HUD between 2010 and 2012, by Martha R. Burt, Molly Scott, from the Urban Institute, Gretchen Locke, Larry Buron, Jacob Klerman, Nicole Fiore from Abt Associates and Lindsey Stillman, from Cloudburst Consulting Group

Subject(s): supportive_services, homelessness_prevention

The researchers noted that while the program had a wide-ranging impact, with innovative program components at certain places, homeless prevention services providers struggled to determine who should receive assistance. Some providers created additional hurdles that might have prevented help for those in need. The data collected could not confirm whether HPRP prevention programs reached those households most likely to fall into homelessness. Thus, the researchers called for improved coordination among antipoverty and homeless services agencies, and additional research that would better understand targeting, impact, individual differences in impacts, and cost-effectiveness of homelessness prevention programs.

Housing for Youth Aging Out of Foster Care

Conducted with funding from HUD, published May 2014, under Mathematica Policy Research and Chapin Hall at University of Chicago

Subject(s): youth_homelessness

For many young people who age out of foster care, the transition to adulthood can be abrupt. At age 18 or 21, these young adults must secure suitable housing—in most cases, with little or no support from either their family or the state. As a result, many youth who age out of foster care find themselves homeless or precariously housed. Across a handful of small, regional studies, researchers estimated that between 11 and 37 percent of youth who age out have experienced homelessness and are more likely to experience precarious housing arrangements. Without stable housing, youth are at greater risk of poor educational, employment, physical health, and mental health outcomes. The NYTD outcomes survey, which follows former foster youth as they age from 17 to 19 and to 21, is expected to enhance understanding of the achievements and housing instability among this population on a large, national scale.

Housing Problems and Needs of Native Hawaiians

Conducted with funding from HUD Policy Development and Research under Maris Mikelsons (the Urban Institute) and Karl Eschback (University of Houston)

Subject(s):

This study aims to assess the housing problems and needs of Native Hawaiians. The study found that almost half of Native Hawaiian households experience housing problems. The researchers also found that the Native Hawaiian population is younger, has lower average education, higher unemployment, and lower incomes than the non-Hawaiian population. One of the major issues faced by Native Hawaiians is overcrowding due to high housing prices, affecting around 37% of households.

Houston, Texas: Community Encampment Report

Conducted during fall 2019

Subject(s): unsheltered_homelessness

In the decade preceding the study, encampments in Houston were "small, densely populated encampments scattered throughout the city, mostly underneath highs and along the bayous." However, despite the declines in the unsheltered homeless population, in 2016 three large encampments emerged in downtown Houston; as of the time of this report, approximately 200 people lived in Houston encampments. The report then goes on to characterize the city's encampments, before characterizing those who live in them. It was found that in two of the encampments — Pierce and Chartres — residents were about 60 percent Black/African American and 70 percent male. About one-half were between the ages of 25 and 50, and one-fourth between the ages of 51 and 60. Many were identified to have significant barriers to housing, including disability, substance use disorder, developmental disability, etc. The report also identified the types of services available to those in encampments, as well as the interactions between those living in encampments and local organizations, such as the police department, the Mayor's Office for Homeless initiatives, and Healthcare for the Homeless.

HUD Investments in Impoverished Areas for House Report 116-106

Conducted with funding from HUD

Subject(s): housing_market

This survey presented the percentage of funds allocated by each HUD program, between fiscal years 2017 and 2020 to two types of impoverished communities: persistent poverty counties and high-poverty areas (census tracts). The survey found that across programs, HUD funding was

“more likely to be used in high-poverty areas than in persistent poverty counties.” The survey also found that HUD funding was more likely to be used in high-poverty areas than in persistent “poverty counties” and the different distribution of funding amongst the programs largely reflects HUD’s formulas to allocate funding based on criterias set by law or the department itself.

Implementing Approaches to Address Unsheltered Homelessness

Conducted with funding from HUD from 2015 to 2019

Subject(s): unsheltered_homelessness, Continuum_of_Care

In an effort to introduce extensive nuance to the effects of housing markets and other factors on the rates of people experiencing unsheltered homelessness, 336 CoCs were grouped into four categories based on their rates of unsheltered homelessness. In order to compare the groups, the research identified differences within housing market factors: home values, median rental prices, and fair market rents. Isolating these variables was important because the importance of other variables was put into question. For example, the rate of new housing permits was consistently higher in communities with “increasing” counts — contrary to what many would assume. The study proposed that this was because newer built homes in tight markets could still be unaffordable for many people at risk of homelessness. “Steady Decreasing” counts consisted of looser markets with smaller shares of renter-occupied units, higher renter-vacancy rates, and lower overcrowding. These two variables are important to observe within different areas to understand developing situations for those at risk of homelessness. Furthermore, the research found that areas with “Fluctuating” count rates received the most resources and programs such as permanent supportive housing and mean bed counts per capita. “Steady Increasing” had the least. Overall, the areas with higher counts of unsheltered homelessness had longer periods of

homelessness than the other groups, indicating low CoC successful exit rates and outreach programs.

Improving the Coordination of Services for Adults with Mental Health and Substance Use Disorders: Profiles of Four State Medicaid Initiatives

Conducted with funding from HUD Office of the Assistant Secretary for Planning and Evaluation (ASPE), between 2013 and 2014, under Mathematica Policy Research

Subject(s): physical_health, mental_health, suppportive_services

States are seeking to address the fragmented delivery of services for Medicaid beneficiaries with behavioral health conditions. This report highlights the approaches adopted by four states--Illinois, Louisiana, Massachusetts, and Tennessee--to improve the coordination of physical health, behavioral health, housing, and other supportive services. It is not an exhaustive review of all state programs that are attempting to improve the coordination of services for this population; previous reports have identified several other states and communities that are adopting innovative strategies (Greenberg 2012; Hamblin et al. 2011; Andrews et al. 2014). These case studies are intended to provide a snapshot of some of the significant features of selected state programs and identify some of the key successes and challenges associated with their strategies, as reported by state Medicaid officials, managed care representatives, providers, and consumers.

Issue Brief: Evaluating Housing Programs for Youth who Age out of Foster Care

Conducted with funding from HUD, in collaboration with HHS, between 2014 and 2015, under Amy Dworsky; and Robin Dion

Subject(s): youth_homelessness

In this policy brief, researchers noticed a lack of evaluation on the effects of housing programs aimed at youth who age out of foster care. Therefore, this paper presents "the importance of evaluating housing programs, [...] the barriers to evaluating them, and how the field can move forward to build the evidence base in this area." According to the brief, an evaluation would involve a quasi-experimental design, measure both housing and non-housing outcomes, and face barriers such as low sample size, reluctance to randomly assign youth to treatment and control conditions, and the need to follow youth over time. However, researchers also emphasize the importance of an evaluability assessment before the evaluation, taking into consideration the need for a logic model linking intended outcomes to program components, the target population and its size, the characteristics of the youth being served, the referral process, the feasibility of random assignment, and the demand for services versus the capacity of the program.

Market Predictors of Homelessness: How Housing and Community Factors Shape Homelessness Rates Within Continuums of Care

Conducted with funding from HUD, published in 2019 with 2015 to 2017 data, under Hiren Nisar; Mallory Vachon; Charles Horseman; and Jim Murdoch

Subject(s): unsheltered_homelessness, housing_market

A key motivator of the study was a desire to understand how larger economic forces affect the landscape of homelessness policy. This is in light of a significant increase in the unsheltered homeless population, which rose by 25% in major cities between 2015 and 2017. The study used various datasets to categorize factors affecting homelessness into five domains: housing market, economic conditions, safety nets, demographic conditions, and climate conditions. These factors

were considered across various geographic dimensions to allow for a deeper understanding. Among these factors, housing was the best predictor of community-level homelessness, which follows the findings of previous studies. This suggests that policy should be best directed at addressing housing costs and the state of the housing market.

Patterns of Benefit Receipt among Families who Experience Homelessness

Conducted with funding from HUD, between 2010 and 2015, under Lauren Dunton with Abt Associates

Subject(s): emergency_housing, supportive_services

Families staying in emergency shelters are connected to benefits programs at similar or greater rates compared to other impoverished families in the same communities. This brief presents evidence that continued housing instability makes families susceptible to either losing or experiencing difficulty with access to public benefits. Additionally, the brief finds that being connected to benefit programs may help families who have been in shelter to avoid returning to homelessness; 20 months after a stay in emergency shelter appears to be associated with lower rates of housing instability. Agencies at all levels of government responsible for benefit programs may want to consider ways to assist families with unstable housing to maintain their benefits and also whether families with repeat episodes of homelessness should be targeted for special assistance in obtaining or maintaining benefits for which they are eligible.

Quality Control for Rental Assistance Subsidy Determinations: Final Income Match Report FY 2013

Conducted with funding from HUD's Office of Policy Development and Research under ICF International on September 5, 2014.

Subject(s): supportive_services

The report examined the earnings of adult household members receiving HUD subsidies. It concluded that the difference between actual earned income and unemployment income and reported incomes cost HUD an annual 315.2 million in overpaid subsidy costs.

Rapid Re-Housing in 2018: Program Features and Assistance Models Understanding Rapid Re-Housing

Subject(s): rapid_rehousing

Rapid re-housing (RRH) "provides temporary assistance to help people quickly move from homelessness into housing. RRH programs vary widely in aspects like funding sources, target populations served (for example, individuals versus families), landlord engagement practices, and duration and amounts of assistance provided. This study characterized three features of RRH design—1) how participants access RRH assistance through coordinated entry systems," 2) how they "subsequently navigate the intake and enrollment process," 3) and to "what extent RRH programs are implementing a progressive engagement approach to housing assistance." Conclusions found three common models of RRH rental assistance: 1) one-time financial assistance, 2) short-term intensive assistance: this provided shorter-term, intensive rental assistance, and 3) longer-term partial or tapered assistance. Often full rental assistance was provided for a shorter period (1-3 months) and then partial assistance was provided for a longer period. HUD funding supports many of these programs, and "they exist more frequently in looser, lower cost housing markets."

Rental Housing Discrimination on the Basis of Mental Disabilities: Results of Pilot Testing

Conducted during a 1.5 year period in between 2013–2014

Subject(s): housing_market

Through a pilot test the researchers sought to find the scope of rental housing discrimination on the basis of mental disabilities. Researchers identified their study as "the first comprehensive examination of discrimination in the rental housing market against people with mental disabilities." Testing was administered in multiple urban rental markets across the United States and was divided into two categories: individuals with mental illness and individuals with intellectual or learning disabilities. Over 1,000 tests were conducted primarily by phone and email. The research team found that those with mental disabilities were 1) less likely to receive a response to their inquiry, 2) less likely to be told an advertised unit was available, 3) less likely to be invited to contact the housing provider, and 4) less likely to be invited to inspect the available unit. Policy recommendations included a broad-based educational initiative to teach housing providers about fair housing rights and a redoubling of efforts by public and private housing, disability, and civil rights organizations to engage and educate those living with mental disabilities on anti discrimination laws and their rights.

San Jose, California Community Encampment Report

This study was sponsored by the Office of the Assistant Secretary for Planning and Evaluation at the U.S Department of Health and Human Services and the Office of Policy Development and Research at the U.S Department of Housing and Urban Development. This report characterizes encampments in San Jose, including those next to freeways, underpasses, railroad trucks, and

sidewalks. It also profiled the people living in those encampments. The report then described the interaction between those living in encampments and organizations such as the housing department, police department, and local Fire/EMS services. Lastly, the report outlines the city and public's response to encampments.

Rural Homelessness

Conducted with funding from HHS'S Office of the Assistant Secretary for Planning and Evaluation and Substance Abuse and Mental Health Services Administration and HUD'S Office of Policy Development and Research under Dr. Marjorie Robertson

Subject(s):

This brief is concerned with the gaps in knowledge around the issue of rural homelessness, and the distinctions between rural homelessness and urban homelessness. Several challenges to assessing rural homelessness are identified, including definition differences and undercounting. Researchers identified characteristics of rural homelessness, including poverty and poor health, as well as causes and possible solutions to address homelessness that affects rural demographics.

Section 202 Supportive Housing for the Elderly: Program Status and Performance Measurement

Conducted with funding from HUD's Office of Policy Development and Research during June 2008 under Barbara A. Haley, Robert W. Gray, Lydia B. Taghavi, Dianne T. Thompson, Deborah Devine, Abdollah H. Haghighi, and Seth R. Marcus

Subject(s): supportive_services

This study assesses whether the program Section 202 Supportive Housing for the Elderly — which provides financial assistance to housing projects serving elderly households — has been effective in meeting the needs of very low-income elderly Americans. The study found that the program produces good quality housing that is rated very highly by its residents and that the program costs are reasonable in relation to costs of other development programs.

Study of Household Transition from the Disaster Housing Assistance Program (DHAP-Katrina): Final Report

Conducted by Larry Buron and Gretchen Locke of Abt Associates for HUD's Office of Policy Development and Research in March 2013

Subject(s): temporary_housing, emergency_housing

The study reported on a sample of 36,000 households that participated in the Disaster Housing Assistance Program (DHAP-Katrina). It found that DHAP-Katrina provided "much-needed housing assistance for a vulnerable population, but for more than half of DHAP-Katrina participants there was still no permanent housing solution without the use of HUD's Housing Choice Voucher program for continued housing assistance four years after the hurricanes." The study concludes that this gap can cause some short-term or emergency disaster recipients to need longer-term or permanent housing assistance after initial disaster assistance and post-disaster assistance would need to include job skill development, stable income from employment, and greater access to cash benefits.

Tacoma, Washington Community Encampment Report

Conducting with funding from HUD during fall 2019

Subject(s): unsheltered_homelessness

From 2015 to 2019 the unsheltered homeless population increased by 84 percent in Pierce County, Tacoma "at least in part because of the tight housing market in the entire Seattle region." Before this time period, researchers believed Tacoma only had small encampments of three to four people. By early 2017, that number grew to 100 individuals. The study characterizes encampments in different parts of the city, such as in urban areas, a public park known as "people's park", and vehicular encampments. The typical resident is described as "an older individual with mental health or substance use conditions or both." Those in encampments generally are not families with children. The study then described the interactions and services that local public and private agencies provide, such as the fire/EMS department and Tacoma Rescue Mission.

The Applicability of Housing First Models to Homeless Persons with Serious Mental Illness

Subject(s): mental_health, Housing_First, substance_abuse

This report looked at the Housing First approach of "providing permanent supportive housing to single, homeless adults with mental illness and co-occurring substance-related disorders." The report studied three programs that used the low demand model to respond to substance abuse among the chronically homeless, with low demand defined as an "approach that addresses the harms caused by risk-taking behavior without forcing climates to eliminate the behavior altogether." Clients enrolled in the study were primarily chronically homeless, with nearly all having a history of mental illness. Three-quarters also had histories of substance abuse, with half abusing substances at the time of the study. The report identified the housing pattern of the clients, categorizing them as those who stayed for the entirety of the program, those that left and

returned intermittently, and those who left permanently or died during the study period. The implications of the study include taking into account clients' housing preferences to influence housing stability, a wide array of supportive services, and an adequate supply of permanent housing.

The Bridge to Family Self-Sufficiency (BridgeFSS) Demonstration

Conducted with funding from HUD, the Food and Nutrition Service (FNS), the MacArthur Foundation, the Annie E. Casey Foundation, and The Laura and John Arnold Foundation between 2010 and 2012, by James A. Riccio and Elisabeth D. Babcock from MDRC and the Crittenton Women's Union.

Subject(s): supportive_services

This paper describes an innovative self-sufficiency program that aims to determine "whether low-income families receiving government housing subsidies can, with the right support, reduce their reliance on a range of government transfer benefits, and improve their overall financial security and well-being." Researchers expect BridgeFSS to be more successful than other programs due to more personalized help from job-coaching staff, who use sector-focused labor market information and tailored financial incentives to find jobs that can sustain families and reward upward progress. It would also provide support for achieving multiple goals, and do so over a sustained period of time by using data-driven management.

The Evaluation of the Neighborhood Stabilization Program

Conducted by Jonathan Spader, Alvaro Cortes, Kimberly Burnett, Larry Buron, Michael DiDomenico, Anna Jefferson, Stephen Whitlow, and Jennifer Lewis Buell of Abt Associates and Christian Redfearn and Jenny Schuetz of the University of Southern California in March 2015.

Subject(s): housing_market, supportive_services

The report examines the housing crisis and the neighborhood stabilization program (a part of the HUD's package to deal with the consequences of the housing market collapse. It was meant to assist communities as they dealt with the communal effects of the wave of foreclosures.)

The report found that the program did not provide evidence of a decline in the prices of houses in the neighborhoods where it was implemented; the report concludes that the impacts of the program's investments would have had to be substantial to arrest the decline in home prices in neighborhoods.

Understanding Rapid Re-Rousing through Participants' Experiences

Conducted with funding from HUD over the course of 15 months from late 2018 through early 2020 under Anna Jefferson; Lauren Dunton; Hannah Thomas; Anna Mahathey; and Jill Khadduri of Abt Associates

Subject(s): rapid_rehousing

This research project focused on the impact and effectiveness of Rapid Re-housing (RRH): this solution consists of providing short-term rental assistance and financial aid to transition homeless people into long-term housing. 16 households were interviewed for the study. The results showed that RRH had both successes and drawbacks. The authors agreed with the principle of RRH but called for major improvements. For example, the study suggests that RRH should "offer larger amounts of financial assistance, still temporary but with a longer duration than current programs typically offer," "stronger linkages to mainstream benefits," and "more clearly communicate the

level of support offered." This would avoid cases of people relapsing into homelessness due to the abrupt end of RRH help or connected problems such as addiction to substances or mental health issues.

Understanding Rapid Re-Housing: What Did We Learn?

Conducted with funding from HUD over four years, under Jill Khadduri; Lauren Dunton; and Scott Brown of Abt Associates

Subject(s): rapid_rehousing

This brief compiles information about Rapid Re-housing programs, offering a review of measured outcomes by research studies and performance measurement systems, a non-experimental analysis of the Family Options Study data, a web survey of all Continuums of Care (CoCs) and all Rapid Re-housing in operation as of 2018, interviews with 30 Re-Housing problem participants in two communities, and ethnographic research with 16 of these individuals and families over a 15 month period.

Unsheltered Homelessness and Homeless Encampments in 2019

Conducted with funding from HUD in January 2019, under the Office of Policy Development and Research

Subject(s): unsheltered_homelessness

This short report noted that on a given night in January 2019, "about 211,000 people across the country experienced unsheltered homelessness" which represents 37% of the total homeless population. Researchers noted that "unsheltered homelessness has increased every year since 2015 to levels not seen since the years immediately following the 2008 financial crisis". Several

factors can explain this rise in unsheltered homelessness. These include "the lack of affordable housing and housing subsidies, insufficient resources for addressing homelessness, and a lack of political will to address homelessness in certain cities" as well as "the shortcomings of the current shelter system and the sense of community in encampments."

Veterans Homelessness Prevention Demonstration Evaluation Final Report

Conducted with funding from HUD between 2012 and 2013, by Mary Cunningham, Jennifer Biess, Dina Emam, Martha Burt from the Urban Institute

This research report presented the results of the Veterans Homelessness Prevention Demonstration (VHPD), a 3-year pilot program specifically geared towards homeless and at-risk veterans and their families, authorized by Congress in 2009. The researchers concluded that VHPD had promising results. At the end of the program, "veterans had low rates of return to homeless shelters in their community, high rates of housing stability, and increased rates of employment and income.". The research also discussed similarities and differences between veteran homelessness and nonveteran homelessness. Both groups needed "short-term help paying for housing and some mix of supportive services." However, veterans had "greater physical and mental health needs and different employment assistance needs [...] had more benefits available to them through the VA, and required service providers who have veteran "cultural competency."

Voices of Youth Count Comprehensive Report: Youth Homelessness in America

Conducted between 2016 and 2018 with funding from HUD's Office of Policy Development and Research (PD&R) and support from Chapin Hall, Arcus Foundation, Ballmer Group

Philanthropy, Bill & Melinda Gates Foundation, Campion Foundation, Casey Family Programs, Dr. Inger Davis, Elton John AIDS Foundation, Liberty Mutual Foundation, Melville Charitable Trust, and Raikes Foundation; project led by Matthew Morton, Amy Dworsky, Gina Miranda Samuels, and Sonali Patel

Subject(s): youth_homelessness, homelessness_prevention

The Voices of Youth Count (VoYC) found that at least 700,000 adolescent minors, or 1 in 30 of the population of 13- to 17-year-olds, and 3.5 million, or 1 in 10 young persons between the ages of 18 and 25, were homeless. Some youth are at higher risk for experiencing homelessness like young parents; Black, Hispanic, American Indian, or Alaska Native youth; and lesbian, gay, bisexual, or transgender (LGBT) youth. Homeless youth suffer from several practical disadvantages working against their ability to achieve housing stability like trauma. Thus, the report called for "national estimates of youth homelessness biennially [...] Funding housing interventions, services, outreach, and prevention efforts in accordance with the scale of youth homelessness, accounting for different needs [...] Building prevention efforts in systems where youth likely to experience homelessness: child welfare, juvenile and criminal justice, and education, [...] Tailoring supports for youth experiencing homelessness in rural communities [...] Improving data and devising strategies to understand and address the disproportionate risk for homelessness among specific subpopulations and increasing resources for the rigorous monitoring and evaluation of interventions".

What Happens to Housing Assistance Leavers?

Conducted by the Urban Institute under Robin E. Smith, Susan Popkin, Taz George, and Jennifer Comey in June 2014.

Subject(s): mental_health, physical_health, supportive_services

This report examined the outcomes of those who left housing assistance; HUD's Moving to Opportunity (MTO) had a final evaluation survey that tracked almost 5,000 public housing families from 1994 to about 10-15 years after their moves. Many families described leaving assistance as eviction, and some described it as frustration. Others left the program because the assistance they received was too small or due to family situations. Households no longer receiving federal assistance received less public assistance compared to households still receiving it. The results were framed as into positive and negative moves: positive leaving was defined as those who left for positive reasons and negative leaving defined as the vice versa, such as those who report being pushed out of the program instead of leaving of their own will. Positive leaders had better housing and neighborhood outcomes, though still reported hardship and high debts. Positive leavers have better physical and mental health. Negative leavers experienced worrying levels of housing hardship and instability. The report suggested "the need for new, targeted approaches to support both households at risk of losing their assistance and those moving toward leaving for positive reasons."

What Happens to Low-Income Housing Tax Credit Properties at Year 15 and Beyond?

Conducted by Jil Khadduri, Carissa Climaco, and Kimberly Burnett of Abt Associates in partnership with Laurie Gould and Louise Elving of VIVA Consulting in August 2012

Subject(s): housing_market, affordable_housing

This study examines the Low-Income Housing Tax Credit, (LIHTC) " a significant source of new multifamily housing for a quarter century."At the time of the study, more than one million units of low-income households were at threat of leaving stock by 2020 due to properties

becoming eligible to leave the program over the years. The study focused on whether owners of older LIHTC programs would continue to provide affordable housing for low-income renters. The study examined the factors that affected owners' decisions to leave the program and the implication of their departures on the rental housing market. The results showed that most properties remained profitable even after the fifteen-year period of the program.

Worst Case Housing Needs: 2015 Report to Congress

Conducted by HUD staffers Barry L. Steffen, George R. Carter, Marge Martin, Danilo Pelletiere, David A. Vandenbroucke, and Yunn-Gann David Yao in April 2015.

Subject(s): affordable_housing, housing_market, supportive_services

This report provides national data on problems facing low-income renting families. The report found that worst-case needs fell in 2015 from 2013 after a record high in 2011; this also amounted to a small decrease in the very low-income renter household bracket (a fall of 2 percent). The report found that worse case needs result from a shortage of affordable rental housing. It concluded that a broad strategy and the federal, state, and local levels was needed to continue to "rebuild the economy, strengthen the market, and provide assistance to those families in most need."

Worst Case Housing Needs of People with Disabilities: Supplemental Findings of the Worst Case Housing Needs 2009 Report to Congress

Subject(s):

This report addresses a gap in previous reports of its kind: the "use of a rough proxy to estimate the number of people with disabilities who have severe housing needs." Instead, this report

provides a more “accurate account of such people,” as well as an assessment of the methodology used to calculate the number. While HUD has collected this data since 1973 — and every two years since 195 — the researchers changed the methodology to ask direct questions on disability to estimate the number of households that “include people with disabilities who experience worst-case needs.” They found that the prevalence of nonelderly people with disabilities was higher among renter households than among owner households. According to the income proxy measure, between 2007 and 2009, the number of worse-case-need households including those with disabilities increased by 100,000. This increase was concentrated among those with very-low income.

Summary of Findings

Literature gathered from the Department of Housing and Urban Development emphasized family and child-focused homelessness. While progress has been made in recent years in reducing chronic homelessness among veterans and other single adults, families experiencing homelessness have often been overlooked. In addition, programs designed for families experiencing homelessness tend to focus on the needs of the mother, paying less critical attention to the needs of children. Therefore, the volume of research on youth homelessness and family unification helps to show that efforts to correct these issues are moving in a promising direction.

The literature points to a variety of risk factors that have come to be recognized as common causes of homelessness, including poverty, unemployment, and lack of affordable housing. These risk factors are ultimately exacerbated by a multitude of personal vulnerabilities including mental illnesses, substance use disorders, trauma, domestic violence, incarceration/justice-system involvement, and personal relationships. Though these services may not prevent homelessness, given they are most often offered after the fact, it is still promising to see that there is a broad scope of research that aims to provide the current status of existing supportive services to some of these issues.

That said, there seemed to be a gap in specific interventions, particularly based on population type. There is no perfect solution for every community, and often what may work in one area may not be as effective in another. There should be more research conducted on specific supportive services in specific areas (rural/urban/suburban). Moreover, it makes sense that a lot of the research that we found from HUD seemed to focus on urban hubs. Larger cities have been known to attract more homelessness for a variety of reasons — including wealth disparities and

larger populations – often leading to greater unemployment rates and more unaffordable housing. In addition, cities are known to have a greater availability of resources and social services, which often attracts those seeking help. As a result, we found that a lot of HUD's focus was spent on looking at these urban, metropolitan hubs and analyzing their attempts at combatting homelessness.

Finally, we found that there was a lot of research that fell into the rapid re-housing category, specifically utilizing the housing first model. Rapid re-housing is a primary solution for ending homelessness. It has been demonstrated to be effective in getting people experiencing homelessness into permanent housing and keeping them there. By connecting people with a home, they are in a better position to address other challenges that may have led to their homelessness, such as obtaining employment or addressing substance abuse issues. A lot of the studies HUD produced focused on the analysis and application of these models. The consensus of the literature is that the Housing First model works, yet many policymakers are making the opposite claim.

Future research could explore how to make these findings digestible to the public and skeptical policymakers. Similarly, there was little research on NIMBYs (Not In My Backyarders), or people who oppose affordable housing development projects in their communities. Exploring how to convince suburban communities to allow housing projects to increase our housing stock could be quite rewarding considering affordable housing continues to become less and less affordable.

Department of Veteran Affairs

Literature

A Brief Values Intervention to Support Veterans in Early Buprenorphine Treatment

Conducted with funding from VA Office of Research and Development, between April 2022 and March 2022, under Edith Nourse Rogers Memorial Veterans Hospital's Victoria Ameral

Subject(s): supportive_services, mental_health

This study seeks to develop a brief, values-based intervention that focuses on helping to improve veterans' interpersonal functioning and community integration during early treatment of Opioid Use Disorder (OUD). There is currently medication for OUD that prevents relapse and overdose, but it does not address the functional challenges veterans face in early recovery.

Assessing an Initiative to Facilitate Long-Term Financial and Housing Stability in Vulnerable Veterans

Conducted with $1,197,060 ($289,625 in 2023) in funding from VA Salt Lake City Healthcare System for the period of October 2021 - September 2025 under Richard E. Nelson.

Subject(s): supportive_services

This study seeks to evaluate the impact of the Shallow Subsidy component of the Supportive Services for Veteran Families (SSVF) program on veteran housing, healthcare costs, and health outcomes. By studying the program —which is implemented in 15 communities in the US — the researchers hope to gain a better understanding of Shallow Subsidy on long-term financial self-sufficiency and to develop policy recommendations to improve the Subsidy program.

Assessing the Impact of a Pre-visit Readiness Telephone Call on Video Visit Success Rates

Conducted with funding from VA Health Services Research and Development under Charlie Wray and Archana Sridhar et al.

Subject(s): supportive_services

During the pandemic, the VA prioritized the use of video-based over telephone-based interactions. This study examines the impact of a pre-visit readiness telephone call on the success of the following video visit. The researchers used the San Francisco VA virtual care clinic to determine this. Some of the patients received a call before their virtual appointment that taught the patient how to log onto the portal and tested audio and video connection, while other patients did not. The study found that the use of a pre-visit telephone call reduced the number of failed video calls by a third, as well as reduced the time providers spent on troubleshooting technical issues. This finding could be helpful for reaching certain populations that have a lower probability of using video-based care.

Capturing the Dynamics of Homelessness through Ethnography and Mobile Technology

Conducted with $1,457,256 ($453,816 in 2023) in funding from Edith Nourse Rogers Memorial Veterans Hospital for the period of August 2020 - July 2023 under PI Donald K. McInnes.

Subject(s): supportive_services

This study explains the instability that exists within homeless veteran populations, discussing how individuals are constantly moving from one place to another (i.e. transitional housing to staying with family to homeless shelter etc.). These transitions cause disruptions that interfere with access to care and treatment adherence, however not enough is known about the transitions themselves – "such as their in-the-moment emotions, behaviors, geographic movements, and social support." As a result, this project aims to (1) characterize the real-time lived experience of

homeless Veterans, including day-to-day activities, interactions with services, and residential transitions using a formative ethnographic approach, (2) refine and tailor smartphone data collection methods and (3) conduct a 4-week demonstration of smartphone data collection to evaluate its acceptability and feasibility with Veterans experiencing homelessness, all to ultimately "improve knowledge of Veterans' trajectories into and out of homelessness and their day-to-day barriers to healthcare and other services."

Comparing Quality of Care between VA-Delivered and VA-Purchased Care: The Need for Better Risk Adjustment

Conducted with funding from VA Office of Research and Development between 2022 and 2026, under VA Boston Healthcare System's Amy Rosen

Subject(s): supportive_services

Veterans are able to access more care in their communities due to the MISSION Act of 2018, but this also led to concerns about the quality of care veterans receive in their communities. This study aims to improve existing risk adjustment methods used by VA with an application for VA and Community Care (CC) healthcare quality comparisons in hopes of allowing veterans to make more informed decisions about where to receive care.

Designing a Mobile App to Support Academic Success for Student Veterans

Conducted with funding from the VA Office of Research and Development, between July 2022 and June 2024, under Erin Reilly

Subject(s): supportive_services, mental_health

Student veterans are more likely to suffer from mental health disorders and experience academic issues (such as lower enrollment rates and slower degree attainment) than civilian students in higher education. This project aims to develop and evaluate a comprehensive educational mobile app for student veterans with mental health disorders, VetEd. The researcher will look at previous research on veterans and education to design features of their app.

Diagnoses Treated in Ambulatory Care Among Homeless-Experienced Veterans: Does Supported Housing Matter?

Conducted with funding from the Locally Initiated Project Grant from the VA LA HSR&D Center of Innovation (LIP 65-029), Veterans Administration's PACT Demonstration Laboratory initiative, and the VA Office of Patient Care Services, between 2010 and 2011, under Sonya Gabrielian

Subject(s): physical_health, mental_health

The research found that veterans who were formerly homeless and now housed through VA Supported Housing (VASH) had a 1.7 to 2.5 times more chance than currently homeless Veterans to receive treatment for chronic physical illness, acute physical illness, mental illness, and substance use disorders. They also have roughly 10% more chance to have multiple health visits to treat these conditions. Therefore, we could conclude that supported housing may reduce disparities in the treatment of diagnoses commonly seen in ambulatory care.

Differences in Experiences With Care Between Homeless and Nonhomeless Patients in Veterans Affairs Facilities With Tailored and Nontailored Primary Care Teams

Conducted with funding from VA Health Services Research and Development Grants between 2014 and 2015, under Audrey Jones and Leslie Hausmann et al

Subject(s): physical_health, mental_health

This research aimed to learn more about how homeless and non-homeless veterans perceived experiences in primary care facilities, depending on whether they had a homeless-focused primary care team (HPACT) or not. The results showed that homeless veterans were more satisfied receiving care in an HPACT facility. This proved that HPACT is having an impact in improving medical care for homeless veterans. However, more research would be needed to assess whether HPACT has a long-term effect on homeless veterans by helping them have better health conditions.

Differential Risk for Homelessness Among US Male and Female Veterans With a Positive Screen for Military Sexual Trauma

Conducted with funding from VA, in 2010, under VA Salt Lake City Health Care System's Emily Brignone et al.

Subject(s): mental_health

Researchers investigated whether military sexual trauma (MST) can be a risk factor for homelessness following veterans' last deployment. The research uses data from veterans who used VHA services from 2004 to 2013. Using measures of 30 days, 1 year, and 5 years, following the first VHA interaction, the research concludes that MST has a significant and independent relationship as a risk factor for homelessness among veterans and that this risk was higher among male homeless veterans. This research warrants homeless prevention strategies

that treat trauma, especially, sexual trauma, to mitigate instances of homelessness in veterans. And, how prevention of sexual abuse could potentially reduce rates of homelessness.

Does housing chronically homeless adults lead to social integration?

Conducted with funding from VA, from 2004 to 2009, under VA Connecticut Healthcare System's

Jack Tsai; Alvin S Mares; and Robert A Rosenheck

Subject(s): supportive_services

Among chronically homeless veterans who participated in the VA's Collaborative Initiative to Help End Chronic Homelessness, researchers sampled 550 participants to measure success of social integration after program entry. While measures of participants' stability in housing did improve, measures of social integration did not change significantly. Engagement in community and community activities had a slight significant increase, but half of activities maintained similar participation levels to baseline. The nature of social relationships did shift during the study, with participants relying less on clergy, and more on service providers. The research demonstrated a need for more focused efforts on social integration, even for those living in supported housing. Further research can also be done in the nature of relationships themselves, and how the types of social integration may transform under supportive housing, if social interaction may become dependent on the in-housing network instead of an outside social network.

Enhancing Community Integration for Homeless Veterans

Conducted with $6,593,900 ($1,408,600 in 2023) in funding from VA Greater Los Angeles Healthcare System for October 2020 - September 2025 under Michael F. Green.

Subject(s): mental_health, physical_health, supportive_services

This study discusses the lack of effort that has been placed in supporting homeless veterans in their integration to a community after being provided housing, explaining its importance in their long-term mental and physical well-being. As a result, researchers have proposed the development of a Rehabilitation Research & Development Center at the VA Greater Los Angeles (GLA) in order to create "an interdisciplinary community of researchers, educators, and clinicians to generate intervention and translational research to improve community integration for these Veterans." They describe three main focus areas for this project, including (1) the development and validation of innovative assessments of factors that predict community integration, (2) the identification of determinants that predict improvements in community integration, and (3) the development and adaptation of these interventions to enhance community integration for homeless and recently-housed veterans.

Evaluating Health Services for Justice-Involved Veterans to Improve Policy and Practice

Conducted with $1,192,368 ($291,505 in 2023) in funding from VA Palo Alto Healthcare System for the period of January 2022 - December 2025 under Andrea K. Finla.

Subject(s): supportive_services, physical_health, mental_health

This proposal seeks to develop a Veteran Justice Program (VJP) evaluation dashboard for justice-involved veterans to analyze gaps in VJP services with greater ease. The goal is for the easier identification of policies and practices to improve the healthcare (physical and mental), mortality outcomes, and housing situations of justice-involved veterans.

Evaluating the impact of dental care on housing intervention program outcomes among homeless veterans

Conducted with funding from the VA's Office of Dentistry and the National Center on Homelessness Among Veterans, between August 2008 and September 2009, under Elizabeth Nunez

Subject(s): supportive_services

The research showed that veterans who received dental care were 30% more likely to complete the homeless intervention program than those who didn't receive care, 14% more likely to be employed or financially stable, and 15% more likely to have obtained residential housing. These results showed that providing dental care has a substantial positive impact on homeless veterans participating in housing intervention programs and program planners should consider the inclusion of dental care in their programs. This research raises additional questions on the possible inclusion of other types of care into homelessness prevention programs.

Evaluating the VA Make-or-Buy Decision in Emergency Care

Conducted with funding from VA Office of Research and Development between 2020 and 2024, under Veterans Admin of the Palo Alto Health Care System, David Chimin Chan

Subject(s): physical_health, mental_health, supportive_services

Due to concerns about access to and the quality of care at VA facilities, the VA has begun to finance care for veterans that are outside of the VA. This study aims to identify how the quality and cost of care for veterans differ between VA and non-VA emergency care services.

Examining racial differences in community integration between Black and White homeless veterans

Conducted under Derek M Novacek et al.

Subject(s): supportive_services

The researchers investigated the racial difference in community integration experience for Black homeless veterans, given considerations of both potential amplified disadvantage (double-jeopardy), but also stronger social support within Black families, presenting competing hypotheses. Results concluded that after participation in the Department of Housing and Urban Development – Veterans Affairs Supportive Housing program (HUD-VASH) Black formerly homeless veterans have significantly better community integration than white counterparts, including psychological health measures related to community integration.

Factors contributing to eviction from permanent supportive housing: Lessons from HUD-VASH

Conducted with funding from VA, the National Center on Homelessness Among Veterans and the U.S. Department of Housing and Urban Development (HUD), and the Office of Policy Development and Research between October 2008 and February 2016 under Ann Elizabeth Montgomery

Subject(s): supportive_services, mental_health, physical_health

This research project, involving both VA and HUD offices, studied the characteristics of veterans who exited permanent supportive housing (PSH) programs, either by eviction or accomplishment of goals. It found that "Veterans with a drug use disorder and those who received inpatient, emergency, or outpatient care related to mental/behavioral health and substance use conditions

proximal to program exit had a greater risk for eviction." while veterans who received "outpatient primary medical care and supportive services" were more protected from eviction. Thus, the research called for more research on how to identify these risk factors, create frameworks that take into account these findings, and find appropriate solutions to mitigate the number of evictions.

Healthcare service utilization for formerly homeless veterans in permanent supportive housing: Do neighborhoods matter?

Conducted with $314,885 in total funding from May 2015 to April 2020 under Michael F. Green

Subject(s): supportive_services

The study samples 711 formerly homeless veterans who entered Los Angeles' HUD-VASH program, and follows whether the placement neighborhood of their housing has an effect on medical care usage and rates. For the following factors, researchers did not find an effect on medical care: percent of residents below the poverty line, percent of unemployed residents, and neighborhood socioeconomic status index. There was an increase in emergency department use with increase of neighborhood public transportation use and fellow veteran population. Additionally, with increases in property vacancies, there were increases in mental health care use.

"Homelessness and trauma go hand-in-hand": pathways to homelessness among women veterans

Conducted from December 2005 - January 2006 under Alison B. Hamilton; Ines Poza; and Donna L. Washington.

Subject(s): physical_health, mental_health

Researchers identified 3 focus groups with a total of 29 homeless women veterans in Los Angeles, with the purpose of investigating pathways to homelessness among women. The study found that female veterans are up to 4 times more likely to become homeless than non-veteran women. The research concludes that there are 5 prevalent factors in homelessness among female veterans: 1) childhood adversity, 2) trauma and/or substance abuse during military service, 3) post-military abuse, adversity, and/or relationship termination, 4) post-military mental health, substance abuse, and/or medical problems, and 5) unemployment. Researchers recommend further holistic approaches that consider these multiple and interconnected factors.

The research shows that there are uniquely female gendered factors in instances of woman veteran homelessness. Therefore, future strategies should accommodate the complexity that gendered context adds to homelessness.

Implementing a Clinical-Community Partnered Intervention to Address Food Insecurity Among High-Risk Veterans

Conducted with $1,167,348 ($229,044 in 2023) in funding from Providence VA Medical Center for the period of October 2020 - September 2025 under Alicia Cohen.

Subject(s): supportive_services

Because food insecurity (FI) is understudied and there is a pressing need to connect FI veterans with community resources, the project aims to "evaluate current approaches to Veteran FI and identify and pilot strategies to integrate VHA-community partnered interventions to address FI into clinical practice."

Intimate partner violence, unhealthy alcohol use, and housing instability among women veterans in the Veterans Health Administration.

Conducted with funding from VA Health Services Research and Development between 2013 and 2014 under Melissa Dichter and Clara Wagner et al

Subject(s): supportive_services

The research looked into whether there is an association between homelessness and psychological and social factors in women veterans. It was found that amongst the studied cohort, recent domestic violence was associated with a higher risk of homelessness. The researchers found no significant correlation between high alcohol use and homelessness. Thus, the research shed light on two possible aggravating factors for homelessness in women veterans. The researchers concluded with the need for programs that address the housing needs of women experiencing domestic violence. There should be more research on a wider sample of women, looking at these and other potential factors surrounding homelessness.

Medical-Legal Partnerships At Veterans Affairs Medical Centers Improved Housing And Psychosocial Outcomes For Vets

Conducted with funding from the Bristol-Myers Squibb Foundation, between 2014 and 2016, under Jack Tsai

Subject(s): physical_health, mental_health

The research looked at medical-legal partnerships that unite legal professionals and VA healthcare providers in helping address legal problems that can affect health and well-being. Researchers found that "veterans who received more partnership services showed greater improvements in housing and mental health than those who received fewer services, and those

who achieved their predefined legal goals showed greater improvements in housing status and community integration than those who did not." This research represents an opportunity to explore partnerships between VA health and other organizations that assist veterans that can reduce homelessness. More research will need to be done to look into partnerships in states not studied in the research and their causal effects.

Medical-legal partnerships to prevent evictions and homelessness among veterans

Conducted with $1,030,739 ($268,751 in 2023) in funding from VA Connecticut Healthcare System for the period of November 2021 - October 2025 under Marc I. Rosen.

Subject(s): supportive_services

This study seeks to investigate the impact of medical-legal partnership and help from lawyers in the community on the housing situations of veterans with housing-related legal issues. The researchers randomly assigned 300 veterans to medical-legal partnership teams or general legal help and aim to identify any differences in the housing situations or mental health between the two groups.

Military Sexual Trauma Among Homeless Veterans

Conducted with funding from VA, in 2010, under VA Palo Alto Health Care System's Juan Pavao et al

Subject(s): mental_health

The researchers conducted a national study of 126,598 homeless veterans who used VHA outpatient care in the year 2010, to estimate the prevalence of military sexual trauma (VST) among homeless veterans and their rate of mental health service use. Of the population studied,

39.7 % of female veterans and 3.3 % of male veterans experienced MST. Researchers found that a MST diagnosis coincided with high rates of mental health visits used. Moreover, MST was associated with increased likelihood of diagnosis with other mental health disorders. The research demonstrates the importance of care that can specifically treat and address sexual trauma in veterans and homeless populations, given that there is a high prevalence among current populations. It also suggests further research into how homeless veterans may have gendered experiences of homelessness.

Neuroinflammation and Oxidative Stress in Veterans with Schizophrenia

Conducted with funding from VA Office of Research and Development, between January 2020 and December 2024, under VA Greater Los Angeles Health Care System's Yvonne S. Yang

Subject(s): mental_health

This study aims to examine levels of neuroinflammation and oxidative stress in the brains of veterans with schizophrenia and relate these processes to important functional outcomes, such as housing status, occupational status, and social relationships, which have been relatively unstudied. Yang proposes to focus on neuroinflammation and oxidative stress because both have been shown to be altered in patients with schizophrenia compared to healthy controls and may be related to the pathogenesis of schizophrenia.

One-year incidence and predictors of homelessness among 300,000 U.S. Veterans seen in specialty mental health care

Conducted with funding from VA, between 2008 and 2012, under Jack Tsai

Subject(s): mental_health, substance_abuse

This research aimed to determine the indicators that would predict homelessness, by studying the 300,000 U.S. veterans that seek mental healthcare. Amongst the sample, 5.6% suffered from homelessness. Results indicated that unmarried or diagnosed with a drug use disorder were more than twice as likely to become homeless; and those who were Black or had annual incomes less than $25,000 were more than one and a half times as likely to become homeless. With these findings, the research established a clear relationship between trauma and homelessness and indicated the necessity of adapting homelessness prevention programs to these specific socioeconomic groups that have compounded difficulties due to their trauma and status in society.

Preventing Homelessness Through the National Call Center for Homeless Veterans: Analysis of Calls and Service Referrals

Conducted with funding from Homeless Programs Office of Veterans Health Administration, from October 2018 to November 2020, under Jack Tsai

Subject(s): supportive_services

This novel research aimed to examine the effectiveness of the National Call Center for Homeless Veterans (NCCHV) and whether it can help homeless veterans. Using data from healthcare records and NCCHV, researchers found that most homeless veterans were referred to local resources and the use of health and social care services increased after NCCHV contacts. The researchers noted that there should be additional research on outcomes after NCCHV contacts. This research shed light on an underappreciated tool that can be helpful to veterans. There can also be additional research on how to increase the effectiveness and characteristics of callers.

Providing intensive addiction/housing case management to homeless veterans enrolled in addictions treatment: A randomized controlled trial

Conducted with funding from VA, from October 2011 to November 2015, under VA Puget Sound Health Care System's Carol A Malte; Koriann Cox; and Andrew J Saxon

Subject(s): mental_health, substance_abuse

The study investigated whether 181 homeless veterans entering substance abuse treatment saw more improvement in intensive addiction and housing case management (AHCM) or the standard VA care with housing support group as a control. No statistically significant difference was found in long-term housing, as both groups produced improvements in participants. Further, the support group control even outperformed the AHCM approach on certain substance abuse measures. Researchers concluded AHCM has no overarching benefits in comparison to the control.

Quality of Mental Health Services for Homeless Veterans in Primary Care Settings

Conducted with $834,331 ($190,426 in 2023) in funding from VA Salt Lake City Healthcare System for the period of April 2021 - March 2026 under PI Audrey Jones.

Subject(s): supportive_services, mental_health

This study acknowledges the lack of existing data regarding veterans empaneled in homeless patient aligned care teams (H-PACTs), information on aspects of mental health and substance abuse (MHSA) services, and the integration of the two. As a result, this research hopes to (1) compare MHSA services quality of care and outcomes for homeless-experienced Veterans empaneled in H-PACTs versus other PACTs in the same facilities, (2) measure levels and features of MHSA services integration for homeless-experienced Veterans in H- PACTs and

other PACTs and (3) evaluate MHSA service practices and barriers in 4 facilities ranked low and 4 ranked high on measures of MHSA services quality for homeless-experienced Veterans.

Risk Factors for Homelessness Among US Veterans

Conducted with $143,806 in funding from VA, from October 2012 to September 2017, under Jack Tsai and Robert A. Rosenheck of VA New England Mental Illness Research Education, and Clinical Center

Subject(s): mental_health, substance_abuse

This report analyzes 31 previous studies to determine that the most influential risk factors for homelessness among veterans are substance abuse and mental illness. Researchers also identify that veterans are overrepresented among homeless populations and are more likely to become homeless than non-veteran civilians. The report also identifies key gaps in research, such as study of female veterans and veteran family units, as well as specific pathways/timelines for veterans falling into homelessness. The review provides support, but also room for improvement, in current VA initiatives and research regarding homelessness. The report also identifies where there is weakness and/or gaps in research that analyze why veterans are disproportionately homeless.

Risk factors for Veteran food insecurity: findings from a National US Department of Veterans Affairs Food Insecurity Screener

Conducted with funding from VA Health Services Research and Development Center of Innovation in Long Term Services and Supports (Grant CIN 13-419) in November 8, 2021 under Alicia Cohen et al

Subject(s): supportive_services

This study examines the prevalence of food insecurity among veterans using data from the VA Corporate Data Warehouse from July 2017 to December 2018. The research team found that of the 3,304,702 veterans screened for food insecurity, 44,298 answered in the positive. Food insecurity was associated more highly with veterans identifying as non-Hispanic/Black or Hispanic. Veterans without VA disability-related compensation and those without stable housing had higher chances of experiencing food insecurity. This also proved true for veterans with diabetes, depression, or PTSD.

Social and Behavioral Determinants of Health in High-Risk Veterans

Conducted with funding from VA Office of Research and Development, between 2021 and 2025, VA Palo Alto Health Care System's Donna Michelle Zulman

Subject(s): mental_health, substance_abuse, supportive_services

This study will look at a previous study of veterans at high-risk for hospitalization and a new survey of a nationally-representative sample of veterans to determine how social determinants of health (SDH) influence clinical, healthcare use, and experience outcomes. SDH includes housing instability, food insecurity, social isolation, and transportation barriers.

Strategies to improve Utilization of Post-overdose Evidence-based Risk mitigation among Non-fatal Overdoses in VA (SUPER NOVA)

Conducted with funding from VA Office of Research and Development, between February 2023 and January 2027 under VA Palo Alto Healthcare System's Elizabeth M. Oliva

Subject(s): mental_health, substance_abuse

This study will characterize the non-fatal overdoses among Veterans Health Administration (VHA) patients and the extent to which they receive recommended post-overdose care. This study is really looking at the effectiveness of the Suicide Behavior and Overdose Report (SBOR), a national clinical note template that standardizes the reporting process of overdose events and is intended to improve care after the events. It is currently mandated only for intentional overdoses but will soon be mandated for all overdoses, which is why it is important to understand if SBOR is improving post-overdose care as intended.

Substance Use Rates of Veterans with Depression Leaving Incarceration: A Matched Sample Comparison with General Veterans

Conducted with no funding from VA, published August 31, 2020 under Nicholas Spence and April Crawford et al

Subject(s): mental_health, substance_abuse

This study compares rates of substance use and negative outcomes between two matched samples of justice-involved veterans (formerly incarcerated veterans) and general veterans, both with depression and a substance use disorder. This study found that formerly incarcerated veterans used substances at a higher rate compared to those not recently incarcerated and that the substance use is predictive of several other negative outcomes including homelessness, use of inpatient substance treatment, and re-incarceration.

“They’re homeless in a home”: Retaining homeless-experienced consumers in supported housing

Conducted with funding from two grants (I21 HX001307/HX/HSRD VA/United States and IK2 HX002089/HX/HSRD VA/United States) under Gabrielan et al.

Subject(s): supportive_services

Researchers sought to identify explanatory factors for "exiters" from VA permanent supportive housing, who are the exceptional among most successful participants. Among interviews with sampled "stayers", "exiters" and staff, 3 potential qualitative factors were identified: intrinsic motivation, mental health, and independent living skills. There were not many demographic differences (race, gender, etc) that became determinants of exit status, except for previous chronic homelessness (56.5%-stayers/76.5%-exiters) and serious mental illness (23.5%-stayers/35.3%-exiters).

The Leading Evaluations to Advance VA's Response to National Priorities (LEARN) Evidence-Based Policy Evaluation Center

Conducted with funding from VA Greater Los Angeles Healthcare System for the period of April 2022 - March 2027 under Kristina Marie Cordasco.

Subject(s):

According to this brief, LEARN "conducts time-sensitive, rigorous and responsive national evaluations, and disseminates evaluation findings, to influence continual improvements in VA policies and programs." The anticipated impacts of the Center include fulfilling the "core requirements of the Foundations for Evidence-based Policymaking Act of 2018" and providing answers and solutions to questions posed by VA officials and policymakers.

The potential for health-related uses of mobile phones and internet with homeless veterans: results from a multisite survey

Conducted with funding from the VA Career Development Award, the National Center on Homelessness Among Veterans, the HIV/Hepatitis Quality Enhancement Research Initiative, and the eHealth Quality Enhancement Research Initiative from 2012 to 2013, under D. Keith McInnes

Subject(s): physical_health, mental_health, supportive_services

This research looked at how mobile phones and the Internet can be embedded in the health approach towards homelessness. Amongst the 106 surveyed Massachusetts veterans, 89% had a mobile phone (one-third were smartphones), and 76% used the Internet. Among those with a mobile phone, 71% used text messaging. Nearly all respondent would welcome text reminders from their health providers and wanted mobile phone outreach to help them set up appointments. Therefore, the research showed possible new avenues for contacting homeless veterans, engaging them in efforts against homelessness and procuring timely help. More research would be needed to learn more about how homeless veterans feel about different types, methods and frequency of online/phone outreach.

The Role of Medications in Predicting Suicide-Related Outcomes and Unintended Death in Older Veterans

Conducted with $1,052,083 in funding from VA Office of Research and Development between July 1, 2015 and present

Subject(s): mental_health, substance_abuse

This study aims to determine the role of medications as prognostic factors in predicting risk of suicide and unintended death in older veterans. With the increasing misclassification of suicides as accidental deaths by drug overdose, understanding the impact of medications also increases understanding of suicide prevention efforts. The researcher used the CSR&D Merit Award project, which includes all veterans who used VA health care services and were initially 50 years or older in FY 2012-2013. If the study does indeed find that certain medications and patterns of use can be used to identify older veterans with a higher risk of suicide, this could have huge implications for clinical care and suicide prevention among veterans.

Using Data Analytics and Targeted Whole Health Coaching to Reduce Frequent Utilization of Acute Care among Homeless Veterans

Conducted with $1,355,999 ($372,917 in 2023) in funding from VA Palo Alto Healthcare for January 2022 - December 2025 under Daniel M. Blonigen.

Subject(s): physical_health, mental_health

This study discusses the impact that acute care utilization contributes to the cost of healthcare for veterans, especially those who are homeless. As a result, this research hopes to compile and review data analytics in order to target the needs of these homeless individuals to decrease their reliance on acute care services, thus reducing financial expenses. They describe their goal as the integration "of Hot Spotter Analytics with Peer Specialists trained in Whole Health Coaching (PS-WHC)" in order to train a "high-value workforce in a patient-centered approach" and reduce homeless veterans' use of acute care.

VA's expansion of supportive housing: successes and challenges on the path toward Housing First

Conducted in 2012 under Erika L. Austin

Subject(s): Housing_First, substance_abuse

The study uses interviews with staff at eight VA facilities to determine challenges and successes with a Housing First approach, in which participants receive secure permanent housing regardless of sobriety status or treatment accomplishment. The study found that some of the major challenges with implementation were coordination with local housing authorities, balancing the workload of case management with housing placements, and finding housing options without sobriety requirements. The study also found signs of strain among staff.

What do Veterans with Homeless Experience Want Us to Know That We Are Not Asking? A qualitative content analysis of comments from a national survey of health care experience

Conducted with funding from VA Health Services Research and Development under Allyson Varley et al

Subject(s):

This study focused on the content of unprompted comments offered by veterans with homeless experience in the margins of the Patient Care Quality-Homelessness survey in hopes of better understanding the needs of veterans and identifying important questions surveys neglect to ask. The Patient Care Quality-Homelessness survey garnered 5,337 responses. Researchers found that 657 respondents — 12% of all the respondents — left unprompted comments, amounting to a total of 1,933 comments. Compared to people who did not leave additional comments,

commentators were more likely to report homelessness and severe chronic pain. The survey question that received the most comments was “Have you felt you wanted or needed to cut down on your drinking or drug use in the last 12 months? If yes, what substance?” with many of the commentators referencing pain control or their path to recovery.

Which Homeless Veterans Benefit From a Peer Mentor and How?

Conducted with funding from VA Health Services Research and Development under Linda Resnik

Subject(s): mental_health, supportive_services

The research looked into the effectiveness of peer counseling, an approach championed in recent years by the VA. It showed that the majority of participants (83% of the full sample) benefited from peer counseling. Peer counseling was more effective in veterans from minority groups and veterans who had more peer visits and minutes of intervention. It was less effective in veterans with post-traumatic stress disorder. Further research would be needed to understand the causes behind these variations and how to improve peer counseling techniques.

Summary of Findings

The studies collected from the Department of Veterans' Affairs mainly focused on supportive services for veterans, community integration, and the impact of mental health and substance abuse disorders on housing insecurity. Overall, veterans are overrepresented in homeless populations and are more likely to experience homelessness than non-veteran civilians. Incarceration also seems to compound this effect, with formerly incarcerated veterans tending to experience substance use disorders which are predictive of homelessness and other negative outcomes like re-incarceration.

In regards to Housing First, which aims to provide housing regardless of sobriety level, research found that there were some challenges with the implementation of this policy due to the need to coordinate with local housing authorities and find housing options without sobriety requirements. Peer mentorship was also explored as a tool for veterans and was found to be more effective for minority veterans as well as veterans with a greater frequency of peer visits and total minutes of peer intervention, but ultimately was less effective for veterans with PTSD. However, a majority (83%) benefited from this intervention.

Most studies examined mental health and substance use disorders and their intersections with homelessness. Trauma, in particular, is a predictor of veteran homelessness, as is being unmarried or being diagnosed with a drug use disorder. Research on female veteran homelessness examined the link between intimate partner violence and substance abuse on homelessness.